

GROUP MEDICAL INSURANCE POLICY 2026

Section: 01 - Introduction

1. OBJECTIVE

To secure employees and their dependents well-being by providing a comprehensive insurance coverage for domiciliary or in patient hospitalization treatment expenses incurred on account of their medical needs.

SCOPE

The policy coverage is limited to expenses incurred within India and only for enrolled dependents for the policy period 01 January 2026 till 31 Dec 2026.

2. ELIGIBILITY

Employee, Spouse and Children Policy (ESC Policy) - Coverage extended to Employee and dependents.

- Employees (primary insured)
 - ✓ All IBM India regular employees (full time and part time), Fixed Time Hires (FTH) and Interns (Niti Ayog)
 - ✓ All active employees are covered from the beginning of employment (default)
 - ✓ Employees covered under the Employee State Insurance Coverage (ESIC) Act
- Dependents:
 - ✓ Male/Female /Third gender
 - ✓ Spouse or partner (includes same gender for LGBT)
 - ✓ Children of primary insured
 - Up to a maximum of 4 dependent children
 - Should be aged between 0 days - 24 years.
 - should be un-married, financially dependent on the employee
 - Differently abled dependent children with 40% or more disability are covered without any age limit, subject to Government disability certificate.
- ✓ Dependent coverage is subject to the enrolment action by the employee.

Parents Policy - Coverage extended to the parents under the parent policy:

- ✓ All IBM India regular (Full time/ Part time) employees, FTH employees with tenure > 12 months will be eligible to enroll their parents.
- ✓ Enrolment must be done within the enrollment window.
- ✓ Parents up to the age of 90 years are covered in the policy.
- ✓ Coverage will be extended to parent(s) above 90 years of age basis continuity of in the preceding policy year
- ✓ Parents coverage is subject to the enrolment action by the employee. Coverage will be extended on Floater or individual basis, based on the selection.
- ✓ Parents premium will be completely borne by the employee.

Note: No member can be covered twice in the policy even if he/she is dependent on multiple employees. Parents in laws are not covered.

Section: 02 – General Information

GENERAL INFORMATION

Insurance Company	TPA (Third Party Administrator)	Policy Period	Enrolment Timelines
Niva Bupa Health Insurance Company Limited	Medi Assist India TPA Pvt. Ltd.	January 1, 2026– December 31, 2026	January 1, 2026 – January 31, 2026 For employees joining after January 1, 2026 – enrolment to be completed within 30 days of DOJ (DOJ +29 days)

ESC Base policy

IBM's Group Medical Insurance Policy provides comprehensive health coverages to employees, spouse and dependent children, with a family floater sum insured of INR 400,000/- covering a wide range of medical expenses such as hospitalization, surgeries, and outpatient treatments. This ensures that employees and their families have access to necessary medical care without bearing the full financial burden.

ESC Top-up Policy:

IBM's Voluntary Top-Up Policy is an additional health insurance option available to employees. This policy allows employees to enhance their existing health coverages up to INR 11 lakhs by opting for extra insurance that covers medical expenses exceeding the limits of their primary health insurance plan at most a most affordable cost.

Parent Policy:

IBM's voluntary parental policy, often referred to as the Voluntary Group Medisclaim Policy for Parents, is a curated health insurance policy that allows employees to opt for a medical coverage specifically for their parents. This policy is designed to cover medical expenses such as hospitalization, surgeries, and other treatments for the parents of IBM employees with multiple sum insured options ranging from INR 1 lakh, 2 lakhs, 3lakhs, 5 lakhs and 7 lakhs.

Section: 03 POLICY DETAILS

For Regular Employees

Sub section 1 – Employee, Spouse and Children (ESC) Base Policy & Voluntary Top-up Policy.

- Covers all IBM India regular employees for a base sum insured of INR 400,000
- Option available to all employees to enhance their sum insured by availing voluntary top-up, the premium for which is entirely borne by the employee
- Owing to the 2 years lock-in on top-up for 2025 & 2026
 - additional coverage selections made in 2025 will continue in 2026, with a provision to further enhance the sum insured in 2026. Such enhancement must be done within the applicable enrolment period.
 - Employees do not have the option to avail the top-up cover in 2026 if they have not opted for it in policy year 2025
 - New joiners (employees joining IBM on or after Jan 1, 2026) can opt for the top-up cover within 30 days of their date of joining
 - Opting out of the top-up plan or reducing the sum insured is not allowed in 2026
- This is a family floater plan, provided for employee's nuclear family (nuclear family is defined as employee, spouse and up to 4 dependent children)
 - there is no restriction on the coverage amount available for each member; it can be availed up to the limit of the sum insured opted, that is, INR 400,000 for base cover or the enhanced cover in case additional cover is opted
- Health insurance benefits have also been extended to the same gender domestic partners of LGBT employees. Employees may enrol their partner during the renewal/enrolment window as spouse.

- Coverage extended to live-in partners. Change in Live in partner is not allowed during the period of 1st Jan 2026 to 31 Dec 2026.
- Enrolment and Addition of Dependents (ESC Policy):
 - All existing employees as of December 31, 2025, will automatically be covered from January 1, 2026 under the base cover of INR 400,000
 - Spouse & Children - insured as of December 31, 2025, will also be rolled over by default to the next policy year w.e.f. January 1, 2026
 - Existing employees will have the option to validate / enrol / update the dependents' details (spouse and children), enhance the family floater cover and/or parental coverage sum insured, or opt out (if they have not availed voluntary top-up coverage in 2025 for the family floater policy) within the enrolment window period (January 1, 2026 - January 31, 2026)
 - ✓ Coverage will be effective from the inception date of the new policy
 - New employees may avail coverage from their date of joining
 - New employees can enrol their dependents, enhance the family floater cover, enrol parent(s) or opt out within 30 days of their joining (including the date of joining, i.e., date of joining + 29 days)
 - ✓ Coverage will be effective from the date of joining of the employee
 - New dependents should be insured within 30 days (date of below mentioned event + 29 days) from the date of the event
 - From date of marriage for newly married spouse
 - ✓ coverage will be effective for the spouse from the date of marriage
 - From date of birth of new born child
 - ✓ coverage will be effective from the date of birth of the new born child
 - From date of legal adoption of a child
 - ✓ coverage will be effective from the date of adoption
 - Employees who wish to renew / enrol themselves and their eligible family member(s) may log in to the Medi Assist portal (<https://portal.mediassist.in/Home.aspx>) using their User ID and password to subscribe. Alternatively, **employees** can also log in to MediBuddy mobile app to enrol. Please scroll down to learn how to complete online enrolment.
 - In-case of employee demise, coverage under the base sum insured will be extended to enrolled dependents (spouse and children) up to 3 months from the date of demise of the employee and an option to port to a retail plan with the same benefits will be provided (subject to the individual underwriting guidelines of the insurance company). The premium refunds pertaining to top-up sum insured and parents' coverage, in case of no claim submitted under these two coverages, shall be made on a pro-rata basis in demise employee FFS.
- Note:
 - It is the responsibility of the employee to declare correct and accurate information regarding the name and date of birth for themselves / their family members on the Medi Assist enrolment portal. If any information provided by an employee is found to be incorrect or false, **this would result in BCG violation**.
 - No member can be covered twice in the policy even if he/she is a dependent of more than one employee
 - Claims will not be accepted for dependents who have not been enrolled on the enrolment portal
 - No payment will be made against claims which are found to be fraudulent in any manner or supported by any fraudulent means or devices whether by the insured person or by any other person acting on his/her behalf.
 - Wherever in this document age is mentioned, it is computed as completed age on last birthday as on the date of commencement of risk for the individual.

Premium (voluntary top up plan)

Additional Sum Insured	Additional Premium (incl. GST, in INR)	OPD limit	Maternity Limit
1,00,000	998.28	15,000	65,000
2,00,000	1,967.06	20,000	70,000
4,00,000	4,768.38	25,000	75,000
6,00,000	7,316	25,000	80,000
11,00,000	12,036	25,000	1,00,000

Premium Co-share (ESC Policy)

- The premium applicable for coverage under the base sum insured (INR 4 Lac) is shared between the employee and IBM
- Premium co-share deduction
 - For regular employee - INR 2,554.42 (inclusive of GST) per annum
 - For FTH employee - INR 1,284.74 (inclusive of GST) per annum
 - Premium deduction will be done from the employee's salary in the month of April 2026
 - The premium will be pro-rated based on the date of joining in case the employee has joined IBM post inception of the policy; the premium deduction will be done in the succeeding month post completion of 30 days from date of joining
 - There will be no premium deduction if an employee chooses to opt out of the policy, in which case, the employee and their dependents (spouse & children) will not be eligible for any benefits specified under this policy
- This premium is eligible for deduction from the taxable income within the defined limits under Section 80 D of the Income Tax Act
- This premium deduction does not require a declaration in the investment module of India Tax declaration/Tax benefits & Form-16, as the deduction will happen automatically from the employee's salary
- The deduction towards premium amounts payable for primary coverage and enhanced coverage for employee and his/her family will be made from the employee's payroll without any prior intimation. The deduction will be done within 90 days of the employee having enrolled his/her dependents on the Medi Assist portal.
- ***After the renewal enrollment window closes (January 1st to January 31st, 2026), for employees joining IBM, the premium will be deducted in the subsequent month.***

Employees who are on LOA (GI, HCAM, US Onsite etc):

Effective January 1st, 2026, IBM India has introduced a new enrolment process for **(HCAM and onsite employees)** Group Medical Insurance Cover (GMC) 2026. This update ensures that employees on assignment (before Dec 31, 2025), including those working globally, can continue to secure IBM's Group medical insurance coverage for their dependents (spouse, children's and parents) in India.

- This year, a dedicated enrollment emailer and communication will be released for HCAM/onsite employees, providing clear instructions and timelines for policy renewal and coverage options.
- The renewal window for GMC 2026 is from December 1, 2025, to December 15, 2025. Employees must review, confirm, and pay for their chosen coverages during this period.
- Base policy premiums must be paid to IBM India within the renewal enrolment window period. Failure to pay will result in coverage cessation and claims denial.
- Voluntary policy premiums must be paid to Niva Bupa within the renewal enrolment window period. Failure to pay will result in coverage cessation and claims denial.

- Employees who opted for top-up coverage in 2025 are required to continue with the 2026 policy and pay the premium co-share. Failure to pay will result in cessation of the family floater coverage.
- Employees who enrolled their parents in 2025 must continue with the 2026 parent's policy and pay the premium. Failure to pay will result in cessation of the family floater coverage.
- Once an employee opts out of the scheme, they and their immediate family (spouse & children) will not be eligible for any benefits under this policy.
- Employees can log in to the Medi Assist portal (<https://portal.mediassist.in/Home.aspx>) or use the MediBuddy mobile app to opt out.
- If an employee opts out, they will only be eligible for hospitalization benefits due to accidents during employment or treatment of occupational diseases, as required by law, with a sum insured of INR 200,000.
- After payment, employees must notify the insurer and share transaction proof for reconciliation. Payment receipts will be issued for tax exemption purposes.
- This year we have created a dedicated FAQ section specifically for HCAM employee.

Opting out of the ESC Policy

- If an employee does not wish to be part of the ESC policy, he/she has the option to opt out of the policy within 30 days of the enrolment start date
- The employee can log in to the Medi Assist portal (<https://portal.mediassist.in/Home.aspx>) or logon to the MediBuddy mobile app to opt out
- Once the employee opts out of the scheme, they and their immediate family are not eligible for any benefits as specified in this policy like corporate buffer, maternity coverage, cancer screening benefit, domiciliary expenses, enhancement of sum insured, family coverage, etc.
- An employee who opts out would only be eligible for hospitalization benefits on account of accident during the course of employment and for treatment of occupational diseases, as required under applicable law for a sum insured of INR 200,000
- Opt out employees, however, will have the option to enrol their parent(s) for coverage as per the terms and conditions of the voluntary parental plan
- An employee who chooses to opt out in policy year 2025, will have the option to re-join / opt in for 2026 policy, as midterm inclusion under the policy is not allowed. Likewise, if an employee has opted for the ESC policy, he/she will not be able to opt out of the policy mid-year.
- Co-pay is not applicable under the opt out policy

Domiciliary Expenses on Outpatient Care (OPD Benefit)

Key attributes:

- This benefit is available
 - ✓ only for ESC policy
 - ✓ only for specialist consultations (allopathic consultation) and investigations prescribed by a specialist i.e., blood tests, X ray, ECG, MRI, CT scans etc. and not for the treatment taken
 - Specialist means doctors having a diploma or a post graduate degree in a clinical subject post completion of MBBS. The coverage is subject to terms, conditions, and exclusions of the policy. Some of the common clinical diploma and post graduate degrees / qualifications are MD, MS, DM, MCH, DGO, DNB, DCH, DPM, D Ortho, DLO, FRCS, MRCP, FRCSC & FRCAS. Some common specialists include:

Cardiologist	ENT Specialist	Neurologist
Oncologist	Gastroenterologist	Pediatrician
Gynecologist & obstetrics	Orthopedics	Nephrologists
Urologist	Ophthalmologist	Endocrinologist
Psychiatric/ Behavioral consultation by a Psychiatrist (MD Psychiatry, or similar degree) is covered. (Psychologist consultation not covered)		

- Medically prescribed physiotherapy referred by specialists (as mentioned in the section) is covered with applicable co-pay
- All screening tests prescribed by a General Physician (MBBS) and chronic disease screenings are covered

- The benefit is also available for home care cover inclusion for chemo & dialysis cases through accredited providers only
- Inclusion of disability (only PWD) cover of INR 10,000 for listed ailments

Locomotor Disability	Low vision	Specific Learning disabilities
Dwarfism	Deafness	Autism
Cerebral Palsy	Hard of hearing	Mental Illness
Leprosy cured persons	Speech & Language disability	Intellectual disability
Muscular Dystrophy	Chronic Neurological conditions	Thalassemia
Acid attack survivor	Multiple Sclerosis	Haemophilia
Blindness	Parkinson's Disease	Sickle cell disease
Multiple disabilities (including deaf, blindness)		

- The OPD benefit has a sub-limit of INR 10,000 under the basic sum insured of the ESC policy and FTH policy
- The employee has an option to enhance the OPD sub-limit up to INR 25,000 by selecting additional top-up. The sub-limit can be enhanced with selection of top-up coverage, applicable as below:

Top-up Sum Insured (In INR)	OPD Limit (In INR)
1,00,000	15,000
2,00,000	20,000
4,00,000	25,000
6,00,000	25,000
11,00,000	25,000

- **Co-payment Applicable under the OPD benefit**

➤ A co-payment of 50% on the admissible claim amount shall apply on each and every claim by the employee and dependents (Spouse and children)

- The OPD benefit **cannot be availed for:**

- Parents policy
- Routine / preventive health check-ups
- Maternity and infertility related expenses including pre- and post-natal expenses
- Non-Allopathic/Non-Specialists consultation; investigation/Medicines & consumables/Routine Health Checks ups and other regular investigation without any specific illness are not covered / will not be payable even if it is prescribed by a specialist allopathic doctor

- The balance under this benefit/sub-limit cannot be carried forward to subsequent year(s). There is no minimum amount for claim.

- **Special provision under Domiciliary Outpatient care (OPD Benefit)**

In event of any incident identified as a workplace incident and if recommended by IBM

- The co-payment (50%) applicable under the benefit shall be waived for the claim and

- 100% of the eligible expenses under consultation, investigation and treatment / medication would be covered up to the sum insured (INR 10,000) under the benefit
- All other terms, conditions under the benefit would remain unchanged

Additional coverage under family floater (ESC Policy)

- Employees who have availed the top-up coverage in 2025 will continue to enjoy the same in 2026 by default, and they will also have the option to enhance their top-up coverage during the 2026 policy year.
- 2-years lock-in (2025&2026) is applicable on the additional coverage option chosen under the top-up plan in 2025
 - If an employee opts for additional coverage in 2025, he/she shall have the option to retain the same coverage or further enhance the sum insured in 2026
 - There is no provision to opt-out of the policy in 2026 once the employee enrolls under the top-up plan in 2025
 - Employees who have not availed top-up in 2025 will not have the option to opt for top up policy in 2026
 - Employees can enhance the sum insured up to a maximum of 15,00,000 (includes the base cover of INR 400,000) by selecting from the available enhancement options of INR 100,000, INR 200,000, INR 400,000, INR 600,000 and INR 11,00,000
- The incremental premium (inclusive of GST) incurred for the enhancement in sum insured will be deducted from the employee's salary. Please refer Medi Assist portal for rate chart of premiums applicable (<https://portal.mediassist.in/Home.aspx>).
- Employees who wish to enrol may log in to the Medi Assist portal (<https://portal.mediassist.in/Home.aspx>) using their User ID and password to subscribe. Alternatively, can also log in to MediBuddy mobile app to enrol. Please scroll down to learn how to complete online enrolment.

Note:

- Any ailment diagnosed / treated during coverage/enrolment window period under the lower sum insured will continue to have the lower sum insured as the maximum cover (for that ailment and all related ailments /complications). This is applicable when the sum insured has been increased during the enrolment period.
- Coverage at any point for any person under the ESC policy cannot exceed INR 15,00,000 even if both employee and the spouse are employees of IBM (dual coverage is not allowed). The policy sub-limits for maternity, domiciliary expenses, etc. would apply.

Mid Term Inclusion (only for ESC Policy)

- Mid Term Inclusions will be allowed only as an exception for employees availing the policy and having a valid reason for not being able to enrol their dependents like newly married spouse, new-born child and adopted child details. Midterm inclusion may be allowed under the following scenarios:
 - New addition to the family (spouse or child) while employee was on an international assignment and looking for enrolment soon after his/her return to home country India (within 30 days of assignment end date / return date)
 - New hires who could not access his/her IBM India mails / network to refer to the welcome emailer with details / join Start@IBM session / due to any other technical issues / laptop delivery / operating from the client location since their date of joining
 - Employee was on LOA/ any other approved long leave for the complete enrolment window period (30 days from the policy start date or from marriage/childbirth/ adoption event date whichever is applicable)
- To accommodate any such request for Midterm Inclusions of spouse /children, the employee needs to have approvals from the People Manager (as per Blue pages) and would need to submit proof of marriage or birth certificate or any other applicable document over email to the [AskHR BOT](#) for active employees and the [IBM Alumni form](#) for non-IBMers for BOM Team to review /approve and take it up for insurer concurrence. The coverage date will be effective from the date of inclusion once approved and endorsed by the insurer.

- Employees returning from LOA can have their dependents enrolled under the policy by putting forth the request for the same internally in IBM by reaching out to their People manager (as per Blue pages), and [AskHR BOT](#) for active employees and the [IBM Alumni form](#) for non-IBMers seeking relevant approvals. The date of coverage inception for the members will be the date of return of the employee from LOA. The request must be made to ibmcare@mediassist.in or [AskHR BOT](#) for active employees and the [IBM Alumni form](#) for non-IBMers, within 30 days from their day of return.
- There is no provision for mid-term enhancement of sum insured under both the policies viz., Employee, Spouse & Children (ESC policy) / Voluntary Topup policy or Parents policy.
- There is no provision for addition of live in partner during the mid of the policy period.

Removal of Dependents (ESC Policy)

Dependents cannot be removed during a policy period except under the following circumstances

- Divorce
- Death of a dependent
- Resignation of the employee
- The company at its sole discretion can ask for supporting documents for addition or removal of dependents from the coverage

All such requests must be shared over email to ibmcare@mediassist.in; within 30 days of the incident date.

In-case of any queries please contact [AskHR BOT](#) for active employees and the [IBM Alumni form](#) for non-IBMers.

Benefits Covered under ESC Policy:

For details, please refer to the relevant section

- Inpatient Hospitalization
- 30 days prehospitalization
- 60 days post hospitalization
- Day care procedures covered - specified list (Refer to the day care procedure section)
- Waiver of Pre-existing disease exclusion
- No waiting period
- No room rent limit or type restriction
- Copayment - 10% for employee and 20% for dependents. In case of death of an employee the co-pay would be waived for the claim
- Congenital internal diseases are covered
- Congenital external diseases are covered
- Gender realignment surgery coverage (refer to the WPATH Protocol document)
- Coverage extended to the same gender domestic partners of LGBT employees, subject to enrolling the partner during the renewal/enrolment window as spouse
- Coverage extended to Live-in partners
- HIV cover added to the main policy in both OP & IP
- Inpatient Behavioural and Psychiatric treatments are covered within the overall Hospitalization limit. Exclusions – De-addiction program & admissions for confinement. Complications from family planning devices where Hospitalization is required (eg: Impacted IUCD)
- Expenses incurred towards resuscitation / revival in death cases, even when treatment is given in a hospital or ambulance will be considered.
- Hospitalization expenses for suicide cases or attempted suicide cases (coverages as per Mental health act 2017)
- Ambulance Limit at INR 2,500 per hospitalization-covers charges from the place of incidence to the hospital.

- Orthopaedic appliances up to 5% of the eligible hospitalization expenses or actuals whichever is lower - Coverage for assistive aids: Expenses for crutches, wheelchairs, artificial limbs & other assistive aids (Defined assistive aids as indicated and prescribed by doctor post-surgery would be covered. However, aids / equipment for durable and long terms use at home would be excluded)
- Oral chemotherapy drug/tablet coverage within the base sum insured/ top-up coverage
- Laser treatment for correction of eye due to refractive error greater than or equal to 7.5 Degree is covered. The procedure would be covered even if the refractive error is less than 7.5 Degree if the surgery is performed for therapeutic reasons like erosions, non-healing ulcers, recurrent corneal erosions, nebular opacities, etc.
- Domiciliary Hospitalization Benefit
- Coverages for ARMD without any sub-limit
- Maternity benefits
- Sterility treatment /IVF treatment /Other fertility treatments coverage under Maternity benefit
- Infertility to be covered twice in an employment tenure. Surrogacy coverage up-to Maternity Limit
- Domiciliary expenses on outpatient care
- Cancer screening tests, based on health screening and risk factor screening
- Critical Illness Buffer
- Additional sum insured Benefits (Top-up)
- Value Added Services

Sub Section 2 – Parents policy

- The maximum age up to which parents can be insured is 90 years
- The parents' premium completely borne by the employee.
- 2-years lock-in period is applicable for parent(s)' coverage in 2025 & 2026
 - Employees who have enrolled their parent(s) in 2025 policy will have continuity of cover for their parent(s) in 2026
 - Employees who did not enrol their parents in 2025 will not have the option to cover their parents in 2026
- Employees have the option to retain the same sum insured for their parent(s) in 2025 & 2026, or they can choose to enhance the sum insured in 2026
- Employees enrolling both parents have the option to choose a common floater sum insured for both parents or an individual sum insured for each parent
- The premium applicable under floater cover will depend on the sum insured selection and age band of the elder parent, while the premium applicable under the individual cover will depend on the age band of each parent and the respective sum insured selected for each
- Employees enrolling only one parent will have to choose from the sum insured options available on an individual basis. The premium applicable will be basis the parent's age band and the sum insured selected.
- Premium (inclusive of GST) applicable towards parental coverage will be completely borne by the employee and will be deducted from the employee's salary
- Premium (inclusive of GST) will be pro-rated from the date of joining until the end of policy year, i.e., December 31, 2026 for new hires (DOJ >= Jan 2, 2026).
- This amount will be eligible for deduction from taxable income within the defined limits under Section 80 D of the Income Tax Act. This premium deduction doesn't require declaration in the investment module of India Tax declaration/Tax benefits & Form-16 as the deduction will happen automatically from the employee's salary.
- The Parents Policy also attracts an additional Third-Party Administrator (TPA) charge of INR 99 per parent per year and an additional goods and services tax on the same which will also be deducted from the employee's salary. Please refer to the Medi Assist portal for rate chart of premium applicable (<https://portal.mediassist.in/Home.aspx>).
- Owing to the two years lock-in period, there will be no change to the premium with change in the age band of the parent(s) in 2026. However, the total premium payable may change if there is a revision to the applicable taxes or an enhancement to the parental sum insured in 2026
- Employees, joining IBM India in 2026, and availing the policy, can choose to cover their dependent parent(s) for a sum insured of either INR 100,000, INR 200,000, INR 300,000, INR 500,000 or INR 700,000
- New hire employees should enrol their parent(s) within 30 days from employee's date of joining (date of joining + 29 days)

- The risk inception date for employees existing on or before December 31, 2025 will be January 1, 2026. For new joiners joining on or after January 1, 2026, the risk inception date will be their date of joining.
- Claims submitted for dependents whose name has not been enrolled on the website will not be processed by the TPA. Further, the claim will not be paid if such claim is found to be fraudulent in any manner or is supported by any fraudulent means or devise whether by the insured person or by any other person acting on his/her behalf.
- In case of a claim being submitted with only one parent enrolled, addition of the other parent is allowed. In such scenario the coverage will be changed from individual to floater and the floater premium for the selected sum insured will be applicable. The employee may choose to enhance the sum-insured; reduction of sum insured after a claim is made during the enrolment window period, is not allowed.
- Note:
 - Any ailment diagnosed / treated during coverage/enrolment window period under the lower sum insured will continue to have the lower sum insured as the maximum cover (for that ailment and all related ailments). This is applicable when the sum insured has been increased during enrolment window period.
 - Parents-in-law cannot be covered in the policy
 - No member can be covered twice in the policy even if he/she is a dependent of more than one employee. If two or more siblings working with IBM India are found to have enrolled their parent(s) more than once under the policy, it will be considered as BCG violation and strict action will be taken.
 - Wherever in this document age is mentioned, it is computed as completed age on last birthday as on the date of commencement of risk for the individual.
 - If parents were enrolled under the 2025 policy, following actions are not allowed:
 - opting out of the parental plan in 2026
 - reducing the sum insured in 2026
 - switching between plans i.e., individual to floater and vice-versa

Employee's share in the Premium (Parents Policy)

- The premium for insuring the parents is payable on an annual basis and will be completely borne by the employee
- Applicable premium amount for coverage of employee's parent(s) will be deducted from the employee's payroll without any prior intimation
- The deduction will be done within 90 days of the employee having enrolled his/her dependents on the Medi Assist portal.
- The employees (DOJ on or after 1st Jan 2026) will have an option under the renewal window, from January 1, 2026, to January 31, 2026, to choose to pay the parent's premium as a lump sum in April 2026, or as 2 equal instalments in April 2026 and May 2026.
- No change to the premium with change in the age band of the parent(s) in 2026. However, the total premium payable may change if there is a revision to the applicable taxes or an enhancement to the parental sum insured in 2026.

Parent Premium (Individual) inclusive of GST					
Age Band	1,00,000	2,00,000	3,00,000	5,00,000	7,00,000
0-45	5,704.78	7,200.21	9,015.81	11,400.66	13,547.01
46-55	9,651.46	12,129.99	14,776.92	18,255.17	21,385.58
56-65	10,587.14	22,609.79	27,939.23	35,625.04	42,734.42
66-70	11,449.28	24,444.37	30,847.05	47,461.51	62,829.88
71-75	12,899.64	25,733.45	33,664.75	50,000.52	65,519.50
>75	15,067.47	27,353.39	44,724.45	80,096.29	1,13,699.54

Parent Premium (Floater) inclusive of GST					
Age Band	1,00,000	2,00,000	3,00,000	5,00,000	7,00,000
0-45	8,372.48	11,071.59	14,757.36	18,648.30	22,150.134
46-55	14,121.72	18,615.09	24,159.18	29,836.08	34,945.29

56-65	15,484.31	34,648.46	45,640.57	58,182.65	69,784.07
66-70	16,740.19	37,454.30	50,385.35	77,500.96	1,02,582.89
71-75	18,852.27	39,427.64	54,984.27	81,645.68	1,06,974.01
>75	22,010.33	41,906.17	73,032.49	1,30,760.91	1,85,602.92

- **Lock-in Period**
 - 2-years lock-in period is applicable for parent(s)' coverage in 2025 & 2026
 - Employees who enrolled their parent(s) in 2025 policy will have continuity of cover for their parent(s) in 2026
 - Employees have the option to retain the same sum insured for their parent(s) in 2025 & 2026, or they can choose to enhance the sum insured in 2026 within the prescribed enrolment window (Jan 1, 2026 to Jan 31, 2026).
 - If parents were enrolled under the 2025 policy, following actions are NOT PERMISSIBLE:
 - opting out of the parental plan in 2026
 - reducing the sum insured in 2026
 - switching between plans i.e., individual to floater and vice-versa

- **Note:** Parents-in-law cannot be covered in the policy. No member can be covered twice in the policy even if he/she is a dependent of more than one employee. If two or more siblings working with IBM India are found to have enrolled their parent(s) more than once under the policy, it will be considered as BCG violation and strict action will be taken.

Addition of Dependent Parents (Parents Policy)

- New employees, joining IBM India, in 2026 are required to make declarations for enrolment of their parent(s) in policy year 2026
- For all existing employees as on Dec 31, 2025, the same selections will continue to hold good till end of policy year 2026.
- Employees have the option to enhance the sum insured in 2026 by paying the additional premium required for the enhanced sum insured option. Opting out of the parental plan or reducing the sum insured is not allowed.
- New employees availing the policy can opt for parent's coverage within 30 days of joining (date of joining + 29 days) by logging on to the Mediassist Portal (<https://portal.mediassist.in/Home.aspx>) or MediBuddy mobile app. Coverage will be effective from the date of joining.

Mid Term Inclusion (Parents Policy)

- Mid-term addition of parents is not allowed under the policy except for the below listed scenarios:
 - Retirement of parent from a job where he / she was covered
 - Demise of the bread earning parent where the other parent was covered (employee is required to submit required documents within 30 days of the demise)
 - Employee returning to India from an offshore assignment (employee is required to submit the enrolment request within 30 days of their return)
- The above additions are basis the terms & conditions, please refer the relevant FAQs for details
- There is no provision for mid-term enhancement of sum insured in respect of existing members under both the policies viz., Employee, Spouse & Children / Voluntary Top-up policy and Parents policy.
- For all such requests for Midterm Inclusions of parents, employee needs to have approval from their People Manager (as per Blue pages) and would need to submit proof or other applicable documents over email to the [AskHR BOT](#) for active employees and the [IBM Alumni form](#) for non-IBMers for BOM Team to review /approve & take it up for insurer concurrence. The coverage date will be effective from the date the inclusion is approved and endorsed by the insurer.

Removal of Dependents (Parents Policy)

Dependents cannot be removed during the policy period except under the following circumstances

- Divorce
- Death of a dependent
- Resignation of the employee
- The Insurance Company at its sole discretion can ask for supporting documents for addition or removal of dependents from the coverage

All such requests must be shared over email to ibmcare@mediassist.in; please contact [AskHR BOT](#) for active employees and the [IBM Alumni form](#) for non-IBMers with relevant proofs / documents (soft copy) within 30 days of the incident date

Benefits Covered under the Parents Policy:

For details, please refer to the relevant section

- Hospitalization Expenses
- Domiciliary Hospitalization Benefit
- Coverage for ARMD with an overall limit of INR 50,000/- per year
- Additional Benefits
- Value Added Services

Sub Section 3 – Hospitalization

- All pre-existing diseases are covered
- Only investigation, tests and diagnosis which requires Hospitalisation are covered. In case there is an active treatment of the disease following the investigation, tests and diagnosis, the expenses of investigation, tests and diagnosis will be covered.
- Pre-hospitalization expenses incurred 30 days prior to hospitalization and post-hospitalization expenses incurred up to 60 days after hospitalization relating to the illness is covered to the extent of insurance coverage available, provided that the ailment is covered under the policy. This is applicable for all eligible treatments other than maternity.
- NO CREDIT WILL BE OFFERED FOR THESE EXPENSES. All pre-hospitalization claims should be submitted only with / after the main hospitalization claim is submitted. Reimbursement of these expenses (both pre and post hospitalization) is possible only on production of complete and detailed bills and documents relating to the same along with a signed claim form.
- Co-payment Applicable on Hospitalization Expenses
 - A co-payment of 10% on the admissible claim amount shall apply on each and every claim by the employee
 - A co-payment of 20% on the admissible claim amount shall apply on each and every claim by the dependents (Spouse, children and parents)
 - The co-payment on admissible claim amount shall not apply in case of death of the employee during hospitalization

1. Domiciliary Hospitalisation Benefit

- Domiciliary Hospitalization refers to medical treatment for a period exceeding three days for such illness/disease/injury which in the normal course would require care and treatment at a hospital/nursing home but is actually taken whilst confined at home in India under any of the following circumstances namely:
- The condition of the patient is such that he/she cannot be moved to the Hospital/Nursing Home, or
- Patient cannot be moved to the Hospital/Nursing Home for lack of accommodation therein.
- However, the Domiciliary Hospitalization benefits shall **NOT** cover:

- Expenses incurred for pre and post hospitalization treatment
- Expenses incurred for the treatment of any of the following diseases:

Asthma	Bronchitis	Chronic Nephritis and Nephritic Syndrome
Diarrhea and all types of Dysenteries including Gastroenteritis	Diabetes Mellitus and Insidious	Epilepsy
Hypertension	Influenza, Cough, and Cold	All Psychiatric and Psychosomatic disorders
Pyrexia of unknown origins for less than ten days	Tonsillitis and Upper Respiratory Tract infections including Laryngitis	Pharyngitis
Arthritis, Gout, and Rheumatism		

- **Note:** When treatment such as Dialysis, Chemotherapy, Radiotherapy, Eye Surgery, Lithotripsy (Kidney stone removal), D&C and Tonsillectomy is taken in the Hospital/Nursing Home and the insured is discharged the same day, the treatment will be taken under the Hospitalization Benefit Section.
- This benefit is within the overall inpatient cover i.e. there is no additional sum insured or sub limit applicable on this benefit
- **Copayment Applicable on Domiciliary Hospitalization Expenses**
 - A co-payment of 10% on the admissible claim amount shall apply on each and every claim by the employee
 - A co-payment of 20% on the admissible claim amount shall apply on each and every claim by the dependents (Spouse, children and parents)
 - The co-payment on admissible claim amount shall not apply in case of death of the employee during hospitalization

2. Medical Advancement Coverage:

Coverage extended for the following subject to the treatment having FDA approval:

- ✓ Stem cell therapy
- ✓ Medically indicated robotics surgeries
 - All robotic neuro surgeries
 - All robotic Onco surgeries
 - All robotic cardiovascular surgeries
 - Total Radical prostatectomy.
 - Bone marrow transplant for cancer

Note: Gene therapy is not covered

Modern Treatment Methods and Advancements in Technologies

Immunotherapy - Monoclonal Antibody to be given as injection	Balloon Sinuplasty	Vaporisation of the Prostate (Green laser treatment holmium laser treatment)	Oral Chemotherapy
Bronchial Thermoplasty	Intra Vitreal Injections	Robotic Surgeries	Stereotactic Radio Surgeries
Uterine Artery Embolization and HIFU	Deep Brain Stimulation	IONM (Intra Operative Neuro Monitoring)	Stem Cell Therapy

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Inpatient Treatment for Psychiatric and behavioural Conditions

- Applicable only for employee, spouse, and dependent children. It is not applicable to the Parents Policy.
- This benefit is unique considering that health insurance products in India typically exclude coverage of psychiatric conditions
- The benefit shall pay for all eligible expenses incurred on inpatient treatment for any psychiatric and behavioural condition
- The coverage is within the overall inpatient cover i.e., there is no additional sum insured or sub limit applicable on this benefit.
- Some exclusions for this benefit are: (1) De addiction programs. (2) Admission for primary purpose of confinement

Co - payment applicable on Inpatient Psychiatric and Behavioural Expenses

- a) A co-payment of 10% on the admissible claim amount shall apply on each and every claim by the employee b) A co-payment of 20% on the admissible claim amount shall apply on each and every claim by the dependents (Spouse, children) c) The co-payment on admissible claim amount shall not apply in case of death of the employee during hospitalization

3 – Maternity Benefits

- This benefit is available only to employee and spouse. It is not applicable to the Parents Policy.
- The limit for maternity benefit for employees availing the policy is up to a maximum of INR 60,000 within the overall inpatient cover. The maternity benefit amount will be restricted to INR 60,000 even if both husband & wife are employees of IBM.
- Maternity limit can be increased up to a maximum of INR 100,000/- by opting for top-up cover as per the below table:

Top-up Sum Insured	Maternity Limit
100,000	65,000
200,000	70,000
400,000	75,000
600,000	80,000
11,00,000	100,000

- Hospitalization for maternity benefit can be availed for up to 4 children
- The insurance plan also provides for pre and postnatal expenses (incurred up to 60 days after maternity) as a part of the maternity benefit. The benefit will include consultations, prescribed medications, and prescribed investigations up to a maximum of INR 10,000 per maternity event. This benefit is a sub limit of the maternity benefit.
- The pre-natal claims should be submitted only with / after the main claim. Reimbursement of these expenses (both pre- and post-natal) is possible only on production of complete and detailed bills and documents relating to the same along with a signed claim form
- In case of any active treatment given to the new-born baby, the expenses will be treated as child expenses (immunization expenses excluded)
- The benefit also covers pre-natal and post-natal expenses for medically terminated pregnancies
- Sterility treatment /IVF treatment /Other fertility treatments are also covered under the policy up to the maternity sub-limit and can be availed twice in an employment tenure
- Surrogacy coverage can be availed up to the maternity limit of INR 60,000.

Co-payment Applicable on Maternity Expenses

- A co-payment of 10% on the admissible claim amount shall apply on each and every claim by the employee for maternity and related claims up to 4 deliveries
- A co-payment of 20% on the admissible claim amount shall apply on each and every claim by the dependents for maternity and related claims up to 4 deliveries
- The co-payment on admissible claim amount shall not apply in case of death of the employee during hospitalization

Infertility treatment benefit

The infertility treatment benefit is extended to the employee and their spouse under the ESC policy, as part of the Maternity coverage. This implies the treatments (inpatient & day care basis only) under the infertility treatment benefit are covered under the policy up to the maternity sub-limit.

Sterility treatment /IVF treatment / other fertility treatments form a part of the infertility treatment benefit, while below is the list of day care procedures covered under the benefit:

Ovarian drilling	Ovarian cystectomy	Therapeutic insufflation of the Fallopian tubes
Tuboplasty	Therapeutic curettage	Endoscopic polypectomy
Myomectomy	Hysteroscopic or laparoscopic biopsy or removal of uterine fibroid	Incision of the scrotum and tunica vaginalis testis
Surgical treatment of a varicocele and a hydrocele of the spermatic cord		
Assisted reproductive procedures like In vitro fertilization (IVF), GIFT, ICSI		

Below mentioned expenses are not within the scope of infertility coverage:

OPD treatments with regards to infertility treatment	Pre & Post hospitalization expenses of infertility treatment taken
Expenses for diagnosis & screening	Any expense for donor screening or compensation

4.Cataract Surgery Benefits

- Cataract surgery is covered under day care procedures
- Conventional procedures /lens (monofocal) are payable under the policy.
- Expenses related to multifocal lenses and toric lenses will not be admissible under this coverage as these lenses are used only for replacement of the spectacles, and any procedure for avoiding usage of spectacles will fall under cosmetic clause of policy terms and conditions; however, claims (cashless/reimbursement) for Multifocal lens will be allowed if insured is under the age of 45 years
- Femto laser is not covered

5.Health Screening Benefit

I. Health screening benefit is available to eligible members on an annual basis unless otherwise specified

II. Biometric Screening

- Combination of Onsite & offsite mode will be used
- Onsite here means IBM Campus and Offsite means identified network hospitals / diagnostic centres.
- Employees who are not able to participate in the onsite biometric screening camps have a choice to get the screening done at identified network diagnostic centres/hospitals and file a reimbursement claim
- BMI, BP, Random blood glucose, total cholesterol, Hb%, peak flow test, manual short HRA from wellness checkpoint will be offered for all employees through onsite biometrics screening camps.

III. This benefit is extended only under the employee, spouse, and children (ESC) Policy. This benefit is not applicable to the Parents Policy. This benefit is applicable to employees only except for Sections H, I & J where children are eligible.

IV. This benefit is within the overall Sum Insured as a sub limit i.e., there is no additional sum insured. Any Claim will be paid from main sum insured.

V. The benefit can be claimed as a reimbursement. No cashless is available for this benefit.

VI. Co-payment Applicable: There is no Co pay applicable on this benefit

VII. The benefit would cover tests for specific screening of conditions only and not for diagnostic purposes, existing medical conditions, treatment, or follow-up treatment

VIII. Following screening tests are also covered (in addition to biometric screening): Depending on the age / gender & risk factor(s), screening may be suggested.

A. Risk factor screening:

Service	Frequency
Smoking Use Screening	Questions and Counselling by Healthcare Professional on every screening visit starting at age 12 or as clinically appropriate
Alcohol Abuse Screening	Questions and Counselling by Healthcare Professional on every screening visit starting at age 12 or as clinically appropriate
Depression Screening	Questions and Counselling by Healthcare Professional on every screening visit starting at age 12 or as clinically appropriate
Family History of cancer, premature cardiovascular disease, and other significant illness	On every screening visit

B. Physical Exam:

Service	Recommendation
Height, Weight, Body mass index (BMI), Waist circumference	Measurement on every screening visit
Blood pressure measurement	Measurement on every screening visit
Vision screening	Measurement on every visit (Snellen chart recommended) on every visit

C. Blood Tests:

Service	Recommendation
Total-cholesterol and HDL-cholesterol measurement (Full lipid profile including fasting Total-cholesterol, LDL-cholesterol, HDL-cholesterol, and Triglyceride measurement is also acceptable)	Check in men 35 years old and above and women 45 years old and above. Start earlier for any adult with any risk factors for cardiovascular disease. Repeat testing every 5 years if normal or more frequent if elevated.

Fasting blood glucose measurement	Measure in adults 40 years old and above or earlier if tobacco use, obesity, family history of diabetes or large for gestational age baby, hypertension, or dyslipidaemia present. If normal repeat every 5 years; more frequent screening is appropriate based on risk factors.
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D. Other tests

One time HIV Screening (Rapid HIV Test by Blood or Saliva)	Test individuals at increased risk or upon request by patient. May repeat based on risk factors.
One time Hepatitis B Screening	Test the following individuals: • Persons born in geographic regions with HBsAg prevalence of $\geq 2\%$ • Unvaccinated persons whose parents were persons not vaccinated as infants whose parents were born in geographic regions with HBsAg prevalence of $\geq 8\%$ • Injection-drug users • Men who have sex with men • Persons with elevated ALT/AST of unknown etiology • Persons with medical conditions that require immunosuppressive therapy • Infants born to HBsAg-positive mothers • Household contacts and sex partners of HBV-infected persons • Persons who are the source of blood or body fluid exposures that might warrant post exposure prophylaxis (e.g., needle stick injury to a health care worker) • Persons infected with HIV
One time Hepatitis C Screening	HCV-testing is recommended for those who: • Currently inject drugs • Ever injected drugs, including those who injected once or a few times many years ago • Have certain medical conditions, including persons: a) who received clotting factor concentrates produced before 1987 b) who were ever on long-term haemodialysis? c) with persistently abnormal alanine aminotransferase levels (ALT) d) who have HIV infection? • Were prior recipients of transfusions or organ transplants, including persons who: a) were notified that they received blood from a donor who later tested positive for HCV infection b) received a transfusion of blood, blood components or an organ transplant before July 1992 • HCV- testing based on a recognized exposure is recommended for: a) Healthcare, emergency medical, and public safety workers after needle sticks, sharps, or mucosal exposures to HCV-positive blood b) Children born to HCV-positive women Note: For persons who might have been exposed to HCV within the past 6 months, testing for HCV RNA or follow-up testing for HCV antibody is recommended.
Tuberculosis screening	Screening is recommended for: • Household contacts and other close contacts of patients with active TB

	• People living with HIV • Current and former workers in workplaces with silica exposure • TB should be considered in people with an untreated fibrotic chest X-ray lesion • In settings where the TB prevalence in the general population is 124/100,000 population or higher
Osteoporosis screening	Preferred screening test is dual-energy x-ray absorptiometry (DEXA scan). Screen women aged 65 or older. Screening in men and in women under age 65 based on risk factors. Repeat testing should be based on risk factors and findings of previous test.

E. CANCER SCREENING TESTS

Service	Recommendation
Cervical cancer screening	Preferred screening test is the Papanicolaou (Pap) smear. Screen women ages 21 to 65 years, every 3 years.
Breast cancer screening	Preferred screening test is mammography. Screen women ages 50 to 75 years every two years.
Colon cancer screening	Screen all adults ages 50 to 75 by: a) Fecal occult blood testing on 3 consecutive stool samples annually b) Flexible Sigmoidoscopy every 5 years, with fecal occult blood testing every 3 years c) Or Colonoscopy every 10 years

F. VACCINATION

Service	Recommendation
Hepatitis B Vaccine	All doses + booster dose

G. Periodic Check-up Guidelines for Children (Paediatrics)

Service	Recommendation
Well child visit for ages 0-3	at the following ages: • 3 to 5 days • 1 month • 2 months • 4 months • 6 months • 9 months • 12 months • 15 months • 18 months • 24 months • 30 months
Ages 3-18	Every year

G. Risk factor screening for children:

Smoking Use Screening	Questions and Counselling by Healthcare Professional) on every screening visit starting at age 12 or as clinically appropriate
Alcohol Abuse Screening	Questions and Counselling by Healthcare Professional on every screening visit starting at age 12 or as clinically appropriate
Depression Screening	Questions and Counselling by Healthcare Professional on every screening visit starting at age 12 or as clinically appropriate

H. Physical Exam for children

Service	Recommendation
Height, Weight, and Body mass index (BMI), waist circumference	Measurement on every screening visit
Blood pressure measurement	Measurement on every screening visit
Vision screening	Measurement on every visit (Snellen chart recommended)

You can submit reimbursement claims for the screening test(s) availed through offsite medium.

The following is the procedure to submit your reimbursement claims for a screening test through the MediBuddy portal (<https://portal.medibuddy.in>):

1. Login to the MediBuddy portal. Click the **Submit claims** button from 'Claims' tile and select submit hospitalisation claim
2. Enter the details of your screening. The form is divided into 3 parts - This would include your Beneficiary Details, Claim Details, and Declaration of Claim Submission. Add your bank details so that the reimbursement amount can be transferred to your account. Remember to raise separate claims for separate screenings.
3. Scan and upload your documents to enable Medi Assist to start processing your claims based on the online submission even before receiving the physical documents. You must mandatorily submit claim form, eligibility declaration, original bill and receipt, and copy of reports of the tests for the claim to be approved after scrutiny of these originals. Remember to upload documents such as ID proof and address proof.
4. Once you have duly filled in the form and saved it, please re-check all the details entered. After a claim form is submitted, you will not be able to make any changes.
5. Retain the scanned/photocopies of all the documents for your reference
6. Kindly take a printout of filled claim form, sign and submit it within 3-4 days from the date of the tests along with all financial original documents for final settlement of the claim. The following original documents should be submitted to Medi Assist for each claim:
 - a. Claim form
 - b. Eligibility declaration
 - c. original bill & receipt
 - d. Copy of reports of the tests
7. The medical team at Medi Assist processes the claim:
 - In case of approval, the amount is reimbursed via NEFT
 - In case your claim is denied, the denial letter is sent to your registered e-mail quoting the reason for denial of your claim

Note:

- You can submit an online claim beneficiary only after the closure of enrolment window period of 30 days.
- The size of each of document should be less than 5 MB.

- It is the responsibility of the employee to declare correct and accurate information regarding the claim and ensure that the claim submitted is genuine. If any information provided by the employee is found to be manipulated, incorrect or false this would result in BCG violation.

Additional Benefits

Ambulance Expenses:

Ambulance expenses for all emergency hospitalizations are covered up to the limit of INR 2,500. Employees can claim ambulance charges only from the place of incidence/home of the patient to the hospital and not the return trip. Ambulance usage on the return trip will be at the cost of the employee.

Expenses towards Appliances:

Cost of appliances is covered as a part of orthopaedic treatment. These include but are not restricted to braces, splints, crutches, wheelchairs, artificial limb etc. These expenses are a part of the hospitalization benefit and are reimbursable up to a maximum of 5% of the total eligible claim amount or actual expense of the appliance, whichever is lower.

06 Critical Illness Buffer

- This benefit is extended only to the employee, spouse, and dependent children. This benefit is not applicable to the Parents Policy.
- The Critical Illness Buffer can only be used once the family floater amount and the additional coverage (if any) taken by the employee have been exhausted; IBM shall extend support of an additional INR 700,000 per policy year for the treatment of the below listed critical illnesses.

Blindness	Cancer	Coronary artery surgery
Heart valve replacement	Kidney failure	Major organ transplant
Multiple sclerosis	Myocardial infarction	Paralysis
Stroke	Surgery of Aorta	Treatment of any injury arising out of road accidents to employees, spouse and children
Coma of specified severity	Motor Neuron Disease	Complications for a Preterm Baby
Defined Life-threatening emergencies related to maternity	Hospitalization arising of Covid which is life threatening/critical condition	Cochlear implants
Any other exceptional life-threatening conditions (disease/condition/injury which MUST be the PRIMARY CAUSE of threat to life of the insured member in a span of one year)		

The provision to consider exceptional life-threatening conditions will be as per the below laid out procedure:

Defined Life-threatening emergencies related to maternity will be covered under the critical illness buffer once the maternity sub limit is exhausted. These defined conditions are

1. Life threatening PPH (PPH)
2. Post-natal sepsis
3. Life threatening Eclampsia
4. Life threatening issues arising out of perforation & sepsis post MTP

Procedure:

- A team of 3 doctors would be constituted by Niva Bupa (all doctors working with Niva Bupa)
- IBM India Ltd would suggest a designated doctor from CHS team who would be the SPOC. The concerned doctor would be contacted by Niva Bupa team if there's any requirement or for inputs. IBM team will not have any role in decision making
- If, any one doctor from Niva Bupa team agrees the consideration of the exception, the case would be considered as approved for exception

- If Niva Bupa doctors team feels the case is ambiguous, they may refer the case to appropriate external consultants for their opinion
- The decision of Niva Bupa doctors' team would be final and binding on all
- Treatments which are not likely to improve the likely survivability significantly would not be considered. Palliative treatments would be excluded
- The condition / diagnosis / treatment for which buffer is requested must be otherwise admissible under the policy terms & conditions
- The treatment must be for a disease / condition / injury which MUST be the PRIMARY CAUSE of threat to life of the insured member in a span of one year
- Approvals of corporate buffer for any exceptional life-threatening condition would be based on the merits of the case and not form precedence for any future claims.
- **Coverage of HIV / AIDS through Critical Illness Buffer: HIV/AIDS** is included in the critical illness list for critical illness buffer.
- Apart from the above list of illness, the buffer amount of INR 700,000 can be utilized for the treatment of any injury arising out of road accidents; this benefit is applicable to employee, spouse and children
- Co-payment Applicable: The critical illness is linked with the additional coverage taken by the employee. The eligibility for critical illness buffer will be determined as per the table below.

Additional coverage amount opted for self and Family	Co-payment (%)	Total Sum Insured (Base + Additional)
11 lakhs	0%	15 Lakh
4 lakhs to 6 lakhs	10%	8 to 10 Lakh
1 lakh to 2 lakhs	20%	5 to 6 Lakh
No additional coverage	30%	4 Lakh

07 Day care Procedures coverage:

Day Care Procedures will include following Day Care Surgeries & Day Care Treatments:

Day Procedure	Description
Microsurgical operations on the middle ear	Stapedotomy
	Stapedectomy
	Revision of a stapedectomy
	Other operations on the auditory ossicles
	Myringoplasty (Type -I Tympanoplasty)
	Tympanoplasty (closure of an eardrum perforation/reconstruction of the auditory ossicles)
	Revision of a tympanoplasty
	Other microsurgical operations on the middle ear under general /spinal anesthesia
Operations on the nose & the nasal sinuses	Excision and destruction of diseased tissue of the nose

Day Procedure	Description
Other operations on the middle & internal ear	Myringotomy
	Removal of a tympanic drain
	Incision of the mastoid process and middle ear
	Mastoidectomy
	Reconstruction of the middle ear
	Other excisions of the middle and inner ear
	Fenestration of the inner ear
	Revision of a fenestration of the inner ear
	Incision (opening) and destruction (elimination) of the inner ear

	Operations on the turbinates (nasal concha)
	Other operations on the nose
	Nasal sinus aspiration
Operations on the eyes	Incision of tear glands
	Other operations on the tear ducts
	Incision of diseased eyelids
	Excision and destruction of diseased tissue of the eyelid
	Operations on the canthus and epicanthus
	Corrective surgery for entropion and ectropion
	Corrective surgery for blepharoptosis
	Removal of a foreign body from the conjunctiva
	Removal of a foreign body from the cornea
	Incision of the cornea
	Operations for pterygium
	Other operations on the cornea
	Removal of a foreign body from the lens of the eye
	Removal of a foreign body from the posterior chamber of the eye
	Removal of a foreign body from the orbit and eyeball
	Operation of cataract

	Other operations on the middle and inner ear under general /spinal anesthesia
Operations on the tongue	Incision, excision and destruction of diseased tissue of the tongue
	Partial glossectomy
	Glossectomy
	Reconstruction of the tongue
Operations on the skin & subcutaneous tissues	Other operations on the tongue
	Incision of a pilonidal sinus
	Other incisions of the skin and subcutaneous tissues
	Surgical wound toilet (wound debridement) and removal of diseased tissue of the skin and subcutaneous tissues
	Local excision of diseased tissue of the skin and subcutaneous tissues
	Other excisions of the skin and subcutaneous tissues
	Simple restoration of surface continuity of the skin and subcutaneous tissues
	Free skin transplantation, donor site
	Free skin transplantation, recipient site
	Revision of skin plasty
	Other restoration and reconstruction of the skin and subcutaneous tissues
	Chemosurgery to the skin
	Destruction of diseased tissue in the skin and subcutaneous tissues
	Incision and drainage of abscess Destruction of diseased tissue in the skin and subcutaneous tissues

	Retinal detachment
	Vitrectomy and Trabeculectomy
Operations on the salivary glands & salivary ducts	Incision and lancing of a salivary gland and a salivary duct
	Excision of diseased tissue of a salivary gland and a salivary duct
	Resection of a salivary gland
	Reconstruction of a salivary gland and a salivary duct
	Other operations on the salivary glands and salivary ducts
Operations on the tonsils & adenoids	Transoral incision and drainage of a pharyngeal abscess
	Tonsillectomy without adenoidectomy
	Tonsillectomy with adenoidectomy
	Excision and destruction of a lingual tonsil
	Other operations on the tonsils and adenoids under general /spinal anesthesia
Operations on the digestive tract	Incision and excision of tissue in the perianal region
	Surgical treatment of anal fistulas
	Surgical treatment of haemorrhoids
	Division of the anal sphincter (sphincterotomy)
	Other operations on the anus
	Ultrasound guided aspirations
	Sclerotherapy etc.
	Ultrasound guided aspirations
	Sclerotherapy etc.
	Therapeutic Ascitic Tapping
	Endoscopic ligation /banding
	Dilatation of digestive tract strictures
	Endoscopic ultrasonography and biopsy
	Replacement of Gastrostomy tube

Other operations on the mouth & face	External incision and drainage in the region of the mouth, jaw and face
	Incision of the hard and soft palate
	Excision and destruction of diseased hard and soft palate
	Incision, excision and destruction in the mouth
	Plastic surgery to the floor of the mouth
	Palatoplasty
	Other operations in the mouth under general/spinal anesthesia
Operations on the breast	Incision of the breast
	Operations on the nipple
	Excision of breast lump /Fibro adenoma
Trauma surgery and orthopaedics	Incision on bone, septic and aseptic
	Closed reduction on fracture, luxation or epiphyseolysis with osteosynthesis
	Suture and other operations on tendons and tendon sheath
	Reduction of dislocation under GA
	Arthroscopic knee aspiration
	Aspiration of hematoma
	Excision of dupuytren's contracture
	Carpal tunnel decompression
	Surgery for ligament tear
	Surgery for meniscus tear
	Surgery for hemoarthrosis/pyoarthrosis
	Removal of fracture pins/nails
	Removal of metal wire
	Joint Aspiration - Diagnostic / therapeutic
Operations on the female sexual organs	Incision of the ovary
	Insufflation of the Fallopian tubes

	Endoscopic decompression of colon
	Nissen fundoplication for Hiatus Hernia /Gastro esophageal reflux Disease
	Endoscopic Gastrostomy
	Laparoscopic procedures eg colecystectomy, appendicectomy etc.
	Endoscopic Drainage of Pseudopancreatic cyst
	Hernia Repair (Herniotomy / hernioraphy / hernioplasty)
	Therapeutic ERCP
Operations on the prostate & seminal vesicles	Incision of the prostate
	Transurethral excision and destruction of prostate tissue
	Transurethral and percutaneous destruction of prostate tissue
	Open surgical excision and destruction of prostate tissue
	Radical prostatovesiculectomy
	Other excision and destruction of prostate tissue
	Operations on the seminal vesicles
	Incision and excision of periprostatic tissue
	Other operations on the prostate
Operations on the testes	Incision of the testes
	Excision and destruction of diseased tissue of the testes
	Unilateral orchidectomy
	Bilateral orchidectomy

	Other operations on the Fallopian tube
	Dilatation of the cervical canal
	Conisation of the uterine cervix
	Other operations on the uterine cervix
	Incision of the uterus (hysterotomy)
	Therapeutic curettage
	Culdotomy
	Incision of the vagina
	Local excision and destruction of diseased tissue of the vagina and the pouch of Douglas
	Incision of the vulva
Operations on the scrotum & tunica vaginalis testis	Operations on Bartholin's glands (cyst)
	Endoscopic polypectomy
	Myomectomy, hysteroscopic or laparoscopic biopsy or removal
	Incision of the scrotum and tunica vaginalis testis
	Operation on a testicular hydrocele
	Excision and destruction of diseased scrotal tissue
	Plastic reconstruction of the scrotum and tunica vaginalis testis
	Other operations on the scrotum and tunica vaginalis testis
	Surgical treatment of a varicocele and a hydrocele of the spermatic cord
	Excision in the area of the epididymis

	Orchidopexy
	Abdominal exploration in cryptorchidism
	Surgical repositioning of an abdominal testis
	Reconstruction of the testis
	Implantation, exchange and removal of a testicular prosthesis
	Other operations on the testis under general /spinal anesthesia
Operations on the urinary system	Cystoscopical removal of stones
	PCNS (Percutaneous nephrostomy)
	PCNL (Percutaneous Nephro-Lithotomy)
	Tran urethral resection of bladder tumor
	Suprapubic cystostomy
Other Operations	Lithotripsy
	Coronary angiography
	Hemodialysis
	Radiotherapy for Cancer
	Cancer Chemotherapy
	Renal biopsy
	Bone marrow biopsy
	Liver biopsy
	Biopsy/Hysteroscopy ? covered if malignancy is confirmed- capped upto Rs10,000/-
	PT Scan
	Excision of cyst/granuloma/lump
	Ascitic/Plueral tapping
	Varicose veins ligation
	Plastic reconstruction of the penis
	Other operations on the penis

epididymis and ductus deferens	Epididymectomy
	Reconstruction of the spermatic cord
	Reconstruction of the ductus deferens and epididymis
	Other operations on the spermatic cord, epididymis and ductus deferens
Operations on the penis	Operations on the foreskin
	Local excision and destruction of diseased tissue of the penis
	Amputation of the penis
	Plastic reconstruction of the penis
OTHER Procedures	Other operations on the penis
	True cut Biopsy
	Endoscopic Foreign Body Removal
	Vaccination / Inoculation - Post Dog bite or Snake bite
	Endoscopic placement/removal of stents
	Tumor embolization
	Aspiration of an internal abscess under ultrasound guidance
	Procedures of Heart and Blood vessels
	Coronary Angioplasty (PTCA)
	Insertion of filter in inferior vena cava
	TIPS procedure for portal hypertension
	Blood transfusion for recipient
	Therapeutic Phlebotomy
	Pericardiocentesis
	Insertion of gel foam in artery or vein
	Carotid angioplasty
	Renal angioplasty
	Procedures of Respiratory System

	Non-adjuvant chemotherapy to be considered under day care treatment
General Treatment	Lengthening of thigh tendons
	Close reduction of Fracture Cases
	Repair of knee joint
Oncology	IV Push Chemotherapy
	HBI-Hemibody Radiotherapy
	Infusional Targeted therapy
	SRT-Stereotactic Arc Therapy
	SC administration of Growth Factors
	Continuous Infusional Chemotherapy
	Infusional Chemotherapy
	CCRT-Concurrent Chemo + RT
	2D Radiotherapy
	3D Conformal Radiotherapy
	IGRT- Image Guided Radiotherapy
	IMRT - Step & Shoot
	Infusional Bisphosphonates
	IMRT – DMLC
	Rotational Arc Therapy
	Tele gamma therapy
	FSRT-Fractionated SRT
	VMAT-Volumetric Modulated Arc Therapy
	SBRT-Stereotactic Body Radiotherapy
	Helical Tomotherapy
	SRS-Stereotactic Radiosurgery
	X-Knife SRS
	Gammaknife SRS
	TBI-Total Body Radiotherapy
	Intraluminal Brachytherapy
	Electron Therapy
	TSET-Total Electron Skin Therapy
	Extracorporeal Irradiation of Blood Products
	Telecobalt Therapy

	Brochosopic treatment of bleeding lesion
	Brochosopic treatment of fistula /stenting
	Bronchoalveolar lavage & biopsy
	Direct Laryngoscopy with biopsy
	Therapeutic Pleural Tapping
Additional Day Care procedures	Thyroplasty Type II
	Uvulo Palato Pharyngo Plasty
	Vocal Cord lateralisation Procedure
	AV fistula – wrist
	URSL with stenting
	URSL with lithotripsy
	Cystoscopy & Biopsy
	Kidney endoscopy and biopsy
	Epidural steroid injection
	VP shunt
	Laser Ablation of Barrett's esophagus
	EUS + submucosal resection
	Infected sebaceous cyst
	Colonoscopy
	Unilateral
	POP application
	Partial removal of metatarsal
	Remove of tissue expander
	Intra articular steroid injection
	Mediastinal lymph node biopsy
	EUA + biopsy multiple fistula in ano
	D&C
	MIRENA insertion
	Hymenectomy (imperforate Hymen)
Oncology	Template Brachytherapy
	Neoadjuvant chemotherapy
	Adjuvant chemotherapy
	Induction chemotherapy

	Telecesium Therapy		Consolidation chemotherapy
	External mould Brachytherapy		Maintenance chemotherapy
	Interstitial Brachytherapy		HDR BrachytherapyPlastic Surgery
	Intracavity Brachytherapy		Construction skin pedicle flap
	3D Brachytherapy		Gluteal pressure ulcer-Excision
	Implant Brachytherapy		Muscle-skin graft, leg
	Intravesical Brachytherapy		Removal of bone for graft
	Adjuvant Radiotherapy		Muscle-skin graft duct fistula
	Afterloading Catheter Brachytherapy		Removal cartilage graft
	Conditioning Radiothearpy for BMT		Myocutaneous flap
	Extracorporeal Irradiation to the Homologous Bone grafts		Fibro myocutaneous flap
	Radical chemotherapy		Breast reconstruction surgery after mastectomy
	Neoadjuvant radiotherapy		Sling operation for facial palsy
	LDR Brachytherapy		Split Skin Grafting under RA
	Palliative Radiotherapy		Wolfe skin graft
	Radical Radiotherapy		Plastic surgery to the floor of the mouth under GA
	Palliative chemotherapy		

8. GENDER REALIGNMENT COVERAGE:

The IBM India GMC policy also covers Gender Realignment expenses under the WPATH protocol. The coverages under this benefit are as below:

Treatment	Type	Covered	Covered Under
Surgery - change of primary or secondary sex characteristics	Male to Female		
	Breasts/Chest - Augmentation mammoplasty (implants/lipofilling)	Yes	Inpatient or Daycare benefits as per policy conditions

Treatment	Type	Covered	Covered Under
	Genital surgery - penectomy, orchiectomy, vaginoplasty, clitoroplasty, valvopathy	Yes	Inpatient or Daycare benefits as per policy conditions
	Facial feminization, liposuction, lipofilling, voice surgery, thyroid cartilage reduction, gluteal augmentation (implants/lipofilling), hair reconstruction.	Yes	Inpatient or Daycare benefits as per policy conditions
	Female to Male		
	Breasts/Chest - Subcutaneous Mastectomy, creation of male chest	Yes	Inpatient or Daycare benefits as per policy conditions
	Genital surgery - Hysterectomy/ovariectomy, reconstruction of fixed part of urethra, combined with Metoidioplasty or Phalloplasty (employing a pedicled or free vascularized flap), vaginectomy, scrotoplasty, implantation of erection and/or testicular prostheses	Yes	Inpatient or Daycare benefits as per policy conditions
	Liposuction, lipofilling, voice surgery, pectoral implants	Yes	Inpatient or Daycare benefits as per policy conditions
Hair removal	Electrolysis	No	
	Laser Treatment	No	

Treatment	Type	Covered	Covered Under
	Waxing	No	
Hormone therapy		Yes	If part of pre and post hospitalization duration expenses as per policy conditions OR part of domiciliary expenses, as per policy conditions
Voice and communication therapy		Yes	Either If part of pre and post hospitalization duration expenses OR part of domiciliary expenses, as per policy conditions
Psychotherapy		Yes	Either If part of pre and post hospitalization duration expenses OR part of domiciliary expenses, as per policy conditions

9. Value Added Services

Niva Bupa is providing a host of value-added services as listed below exclusive for IBMers

Master Health Check-up, diagnostics, and consultation at defined Network Centres

- You can get discounts up to 20% for a master health check-up for yourself and your immediate family (enrolled in the IBM medical insurance policy). You can also avail discounts on diagnostics and Discounts on consultation at defined Network Centres. The discounts will differ from one network centre to another.
- To avail the benefit, please reach out to Niva Bupa on 9599133153 or write to ibmvalueadds@nivabupa.com. The Niva Bupa team will help you locate the nearest network centre.
- Please show your Medi Assist Health id card (E-card) with Niva Bupa logo or IBM Id card to avail these benefits at the network centre. In case you have any concerns, do escalate by calling on the 9599133153 and a Niva Bupa team member will address it on priority.

Discounts on Apollo Pharmacies:

- There is 11.5% discount offered to IBMers on medicines, 5% discount on purchase of non-pharma (FMCG) item and 15% discount on Apollo private labels products from Apollo Pharmacies. This discount is not available at pharmacies located in the Apollo Hospitals and Apollo Clinics. (the discount is subject to the service providers negotiated package and may vary as per the agreement)

Process:

1. Please carry Talent ID card and show it before billing
2. Use below code for availing discount

<u>PRG ID/Code</u>	<u>PRG NAME</u>
<u>2197</u>	<u>IBM INDIA</u>

Important:

According to the "Drugs Price Control Order" (DPCO) the pricing structure of certain drugs would be regulated by the Government of India.

All pharmacies and chemists have to mandatorily comply to this rule of not extending discounts on DPCO drugs. Hence Apollo Pharmacy will not be able to extend any discount on DPCO drugs. Discounts on rest of the products remains unchanged.

Medical Second Opinion

- Niva Bupa may suggest doctors to IBMers should they seek a medical second opinion. The service will be managed on a case-to-case basis
- You will be required to share the required information in the below table with Team Benefits:

<u>Patient Name</u>	<u>Patient's Location where second opinion is required</u>	<u>Department</u>	<u>Ailment</u>	<u>Patient Contact No.</u>
-	<u>e.g., South Delhi, North Delhi, Gurgaon etc.</u>	<u>e.g., Medicine, Surgery, Neuro, Cardiac etc.</u>	-	-

- The suggestion shall be received by you within 3 days.

- You may then choose from the options provided and schedule a visit for second opinion; the consultation expense will be borne by you.

Section: 04 FIXED TIME HIRES (FTH)

	With tenure > 12 months	With tenure <=12 months
Coverage / Benefits	- Can avail benefits similar to that of regular employees - Employee, Spouse and Children coverage - Base coverage of INR 4 lacs (ESC policy) - Voluntary Opt-out option under ESC policy - Voluntary top up option - Voluntary parents' coverage option - Hospitalization expenses - Domiciliary hospitalization benefit - In patient treatment for psychiatric and behavioral conditions - Domiciliary expenses on outpatient care - Maternity benefits - Health screening benefits - Additional benefit - Critical illness buffer - Value added services Please refer to the relevant sections in previous pages of Regular employees for complete information on above listed benefits.	- Covered for base coverage of INR 4 Lacs with the following benefits: - Employee, Spouse and Children coverage - Base coverage of INR 4 lacs (ESC policy) - Voluntary Opt out option under ESC policy - Hospitalization expenses - Domiciliary hospitalization benefit - In patient treatment for psychiatric and behavioral conditions - Domiciliary expenses on outpatient care - Maternity benefits - Health screening benefits - Additional benefits - Critical illness buffer - Value added services Please refer to the relevant sections in previous pages for the complete information on above listed benefits.
Enrolment	- New employees, joining IBM India in 2026, can enroll their dependents (spouse, children and parents), opt for additional coverage or opt-out of the policy: - Within 30 days of policy inception for existing employees (January 1, 2026 – January 31, 2026) - Within 30 days of date of joining (date of joining + 29 days) for new hires - All FTH employees will be automatically enrolled under the base coverage. - To enroll dependents, enhance sum insured, opt for parental coverage, or opt out of the policy, FTH employees are required to log in to https://portal.mediassist.in/Home.aspx. The login credentials for the initial login on the Mediassist Portal will be as follows: - Username: Talent ID@IBM Password: Date of birth followed by Talent ID (eg: ddmmyyyyTalent ID) - For example, if your Talent ID is 123456789, your username would be 123456789@IBM and if your date of birth is 30-November-2014, your initial password would be 30112014123456789. Please change your password after you log in for the first time. - Please note, FTH employees can now enroll through the MediBuddy app as well. To know more about the same, please refer to the FAQs section.	- Employees can enroll their dependents (spouse & children only), or opt-out of the policy: - Within 30 days of policy inception for existing employees (January 1, 2025 – January 31, 2025) - Within 30 days of date of joining (date of joining + 29 days) for new hires - All FTH employees will be automatically enrolled under the ESC policy. - To enroll dependents, the FTH employees are required to log in to https://portal.mediassist.in/Home.aspx. The login credentials for the initial login on the MediBuddy Portal will be as follows: - Username@IBM Password: Date of birth followed by Talent ID (eg.: ddmmyyyyTalent ID) - For example, if your Talent ID is 123456789, your username would be 123456789@IBM and if your date of birth is 30-November-2014, your initial password would be 30112014123456789. Please change your password after you log in for the first time. - Please note, FTH employees can now enroll through the MediBuddy app as well. To know more about the same, please refer to the FAQs section.

2 Years Lock-in on voluntary plans	- Employees who had enrolled their parent(s) in 2025, will be able to maintain continuity of the coverage, or to enhance their sum insured in policy year 2026 - Owing to a 2-years lock-in period, the top-up selections and parents' enrolments done in 2025 will be carried forward to policy year 2026. - Employees who have not opted for top-up and/or parents' coverage in 2025 will not have the option to avail it 2026.	Voluntary Top-up plan and Parents plan are Not Applicable for FTH employees <= 1 year.
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Fixed Time Hire: Co-share of premium (ESC policy)

- The premium for the base sum insured (INR 4 Lacs) is shared between the FTH employee and IBM
 - A deduction of INR 1,284.74/- (including GST) per annum is applicable towards coverage under the base cover of the policy; this will be deducted from the employee's salary
 - For new hire FTH employee, this amount will be pro-rated basis Date of Joining (DOJ) and will be deducted in the succeeding month after completing 30 days from their DOJ (date of joining + 29 days)
 - Unless an employee chooses to opt out of the policy, the premium co-share for this coverage will be automatically deducted for all FTH employees who were on the rolls of the company as on December 31, 2025
- Note:** FTH tenure is considered as per the offer letter start date and end date

Section: 05 – Interns Policy

- All interns (age group 15 to 18 years), under the Niti Aayog program, will be automatically enrolled under the policy for a base cover of INR 4 Lacs (ESC Policy)
- The policy coverage will be for the tenure of the internship
- No co-pay is applicable for claims arising out of workplace incidents
- The benefits include:
 - Hospitalization expenses
 - Domiciliary hospitalization benefit
 - In patient treatment for psychiatric and behavioural conditions
 - Domiciliary expenses on outpatient care
 - Health screening benefits
 - Additional benefits
 - Critical illness buffer
 - Value added services

Please refer to the relevant sections in the previous pages for details of benefits listed above.

All interns who will covered under the intern policy will not be eligible for opting in for Voluntary Plan (Top Up and Parental Plan)

Sections: 06 Exclusions under the policy

The Insurer will not make any payment for any claim in respect of any Insured Person directly or indirectly for, caused by, arising from or in any way attributable to any of the following unless expressly stated to the contrary in this Policy:

- Invasion, act of foreign enemy, civil war, public defence, rebellion, revolution, insurrection, military or usurped acts, chemical and biological weapons
- Any Insured Person committing or attempting to commit a criminal or illegal act, while sane or insane.

- iii. Any Insured Person's participation or involvement in naval, military or air force operation, racing, diving, aviation, scuba diving, parachuting, hang gliding, rock or mountain climbing.
- iv. The abuse or the consequences of the abuse of intoxicants or hallucinogenic substances such as drugs and alcohol, including smoking cessation programs and the treatment of nicotine addiction or any other substance abuse treatment or services, or supplies.
- v. Obesity or morbid obesity or any weight control program, where obesity means a condition in which the Body Mass Index (BMI) is above 29 and morbid obesity means a condition where the BMI is above 37.
- vi. Alzheimer's disease: general debility or exhaustion ("run down condition"); stem cell implantation or surgery which is not approved by FDA; or growth hormone therapy; sleep apnea, gene therapy, Parkinson's Disease for parents
- vii. Venereal disease, sexually transmitted disease, or illness; (except HIV, which is covered in both OP & IP)
- viii. Vicarious pregnancy, birth control, contraceptive supplies or services and complications arising therefrom.
- ix. Dental treatment and surgery of any kind, unless requiring Hospitalization.
- x. Treatment and supplies for analysis and adjustments of spinal subluxation, diagnosis and treatment by manipulation of the skeletal structure or for muscle stimulation by any means (except treatment of fractures and dislocations of the extremities).
- xi. Circumcision (not if required as a part of treatment of or for a disease or due to injury).
- xii. Laser treatment for correction of eye due to refractive error less than 7.5; if the procedure is performed only to get rid of spectacles or contact lenses the claim is not payable; if the Lasik Surgery is performed for therapeutic reasons like erosions, non-healing ulcers, recurrent corneal erosions, nebular opacities, etc it is payable.
- xiii. Aesthetic or change of life treatments of any description such as treatments to do or undo changes in appearance or carried out in childhood or at any other times driven by cultural habits, fashion or the like or any procedures which improve physical appearance.
- xiv. Plastic surgery or cosmetic surgery unless necessary as a part of medically necessary treatment certified by the attending Medical Practitioner for reconstruction following an Accident or Illness.
- xv. Experimental, investigational, or unproven treatment, devices and pharmacological regimens, or measures primarily for diagnostic, X ray or laboratory examinations or other diagnostic studies which are not consistent with or incidental to the diagnosis and treatment of the positive existence or presence of any Illness for which confinement is required at a hospital.
- xvi. Convalescence, cure, rest cure, sanatorium treatment, rehabilitation measures, private duty nursing, respite care, long term nursing care or custodial care.
- xvii. Any non-allopathic treatment, except Ayurveda treatment Benefit. Expenses incurred on treatment taken under Ayurveda, subject to amounts specified in the Schedule of Benefits.
- xviii. All preventive care, vaccination including inoculation and immunizations, any physical, psychiatric, or psychological examinations or testing during these examinations; enteral feedings (infusion formulas via a tube into the upper gastrointestinal tract) and other nutritional and electrolyte supplements, unless certified to be required by the attending Medical Practitioner as a direct consequence of an otherwise covered claim.
- xix. Charges related to a hospital stay not expressly mentioned as being covered, including but not limited to charges for admission, discharge, administration, registration, documentation, and filing.
- xx. Items of personal comfort and convenience including but not limited to television, telephone, foodstuffs, cosmetics, hygiene articles, body care products and bath additives, barber or beauty services, guest services as well as similar incidental services and supplies, and vitamins and tonics unless vitamins and tonics are certified to be required by the attending Medical Practitioner as a direct consequence of an otherwise covered claim.
- xxi. Treatment rendered by a Medical Practitioner which is outside his discipline or the discipline for which he is licensed; referral fees or out station consultations; treatments rendered by a Medical Practitioner who shares the same residence as an Insured Person or who is a member of an Insured Person's family, however proven material costs are eligible for reimbursement in accordance with the applicable cover.
- xxii. The provision or fitting of hearing aids, spectacles or contact lenses including optometric therapy, any treatment and associated expenses for alopecia, baldness, wigs, or toupees, medical supplies including elastic stockings, diabetic test strips, and similar products.
- xxiii. Any treatment or part of a treatment that is not of a reasonable cost, not medically necessary; non-prescription drugs or treatments.
- xxiv. Artificial limbs, crutches or any other external appliance and/or device used for diagnosis or treatment.
- xxv. Immunization.
- xxvi. Treatment for C3R (CORNEAL COLLAGEN CROSSLINKING WITH RIBOFLAVIN) and INTACS are not payable. Treatment related to ROP (retinopathy of prematurity) and RFL (Retrolental Fibroplasia) is not payable.

xxviii. Quantum Magnetic Resonance Therapy or RFQMR (Cytotron) treatments are not admissible under the policy.

xxviii. Parkinson's Disease for parents are not admissible under the policy.

Some common scenarios where claim is not payable under the policy.

- Any Hospitalization for infusion of only oral medication and intramuscular injection throughout the course of hospitalization then the claim would not be admissible even if there is admission for more than 24 hours.
- Any Hospitalization for less than 24 hours other than specified in the day care list would not be admissible under the policy.
- Any OPD treatment or treatment possible at home or treatment done at home would not be considered under the hospitalization benefits or inpatient benefits of the policy terms and conditions.
- Any OPD consultation or tests related to maternity or infertility would not be covered under the OPD benefits of the policy terms and conditions. (Procedure, investigation, test, consultation which are done to check or increase the chances of pregnancy will be considered under the infertility treatment.)
- Expenses related to any procedure or medicines would not be covered under the OPD benefits of the policy.
- Parents are not eligible for availing the OPD benefits of the policy.
- Expenses related to treatment of Parkinson's disease are not covered for parents under the policy.
- Expenses related to any day care surgery/procedures which are not listed in day care list of the policy would not be covered under the policy.
- Any cosmetic procedure or cosmetic treatment or any procedure which enhances the physical appearance will not be admissible under the policy.
- RFQMR is not payable under the policy
- Any Dental treatment which does not require hospitalization would not be admissible under the policy irrespective of etiology/Cause.
- Any expenses related to the doctor treating out of his discipline then the same would not be considered under the policy.
- Ayurvedic treatment other than the govt prescribed procedures would not be admissible under the policy.
- Ayurvedic treatment on outpatient basis, Admission for routine Panchakarma without diagnosis etiology and treatment plan are not admissible under the policy .
- Ayurvedic Treatment taken at SPA, Resort and non-registered hospital are not admissible under the policy.
- CAPD expenses are not admissible under the policy however CAPD device cost can be admissible.
- Hormonal Therapy is not admissible under the policy.
- Treatment for Morbid obesity and complication or any treatment like Liposuction which would enhance the physical appearance is not admissible under the policy.
- Expenses related to cosmetic treatment would not be admissible under the policy.
- Asymptomatic covid claims are not payable under the hospitalization/inpatient benefits of the policy terms and conditions.
- Claim would not be payable if insufficient documents are submitted
- Family planning procedures are not covered under the policy
- Voluntary termination of pregnancy, faetal Reduction (in case of twins and triplets etc), abortion within 12 weeks of pregnancy are not covered under the policy
- Laser treatment for eye correction having refractive error lesser than +/-7.5 are not covered under the policy
- Implantable contact lens, toric lens and multifocal lens are not payable under the policy.
- Pre and post hospitalization expenses not related to the diagnosis of the main claim would not be admissible under the policy
- Regular Health check-up are not admissible under the policy terms and conditions even if it is prescribed by a specialist.
- Any misrepresented claim would not be admissible under the policy

Note: Above scenarios are only common scenarios of rejections and for complete details regarding the claim admissibility or rejection please refer the complete W3 document.

NOTE: All types of non-medical expenses other than the insurer agreed expenses incurred during the course of hospitalization are not covered and have to be paid to the hospital before discharge.

Section: 07 – FAQ's:

1. Frequently Asked Questions (Regular)

Q. Who is my insurer?

A. Niva Bupa Health Insurance company limited is the insurer for policy year 2025 and 2026.

Q. How does family floater work?

A. The sum insured opted for at the time of enrolment can be utilized by any member of the family, that is, self, spouse & children up to the sum insured limit. There is no restriction on the size of the individual claim in a year as long as the family does not exceed the limit of INR 400,000 or the enhanced cover in case you have opted for the enhanced (Top-up) cover.

Q. Who can be the eligible beneficiaries under the policy and the respective coverage limit?

A. You and your nominated dependents (spouse and children*) (**upto 4 children**) are eligible under a family floater cover of INR 400,000 unless you have opted out of the policy. If you wish to enhance the coverage beyond the family floater of INR 400,000 (up to INR 1,500,000 (**11,00,000 Top-up + 400,000 Base coverage**) in total by opting in the voluntary TOP UP sum insured), the incremental premium has to be borne by you**.

You have the option of covering your parents under the parents' policy, but the premium, TPA charges and the goods and services tax would have to be borne by you. Please note parents-in-law cannot be covered in the policy.

NOTE: No other dependents can be insured under this health plan.

* up to 4 dependent children are covered up to age of 24. However, the policy has no age cap for a child suffering from any physical disability subject to the employee submitting the disability certificate given by competent authority

** Premium chart is available on the site <https://portal.mediassist.in/Home.aspx>

Q. Who is Medi Assist India TPA Private Limited?

A. Medi Assist India TPA Private Limited is your service provider who will facilitate administration of IBM India Limited Group Medical insurance Policy (GMC) on behalf of Niva Bupa Health Insurance Company Limited and assist you in accessing quality health care. It is not an insurance company; it acts as a liaison between IBM and the insurance company.

Q. What services can I access through Niva Bupa /Medi Assist India TPA Private Limited?

A. The following services are available to employees

- Online Enrolment System: For self and dependents
- Mobile app named MediBuddy
- Electronic id cards: For self and dependents
- Network Hospital: The largest network of hospitals in the country
- Preferred Network Hospital: Discounted package rates on treatments
- Cashless hospitalization facility: For treatment at network hospitals for ailments covered under the Group Mediclaim Policy
- Claims administration services: Registration of each claim
- Assessment of each claim for eligibility under the plan
- Recovery of missing documents if required
- Submission of claim to the TPA and follow up for speedy reimbursement
- Dedicated Helpline - Both Voice and mail-based services

Q. What is MediBuddy app?

A. MediBuddy is a mobile app introduced by Medi Assist. It empowers you by providing on the go access to enrolment, ecards, network hospitals, claims and other benefits.

Q. Where can I download MediBuddy app?

A. Download MediBuddy app from your app store - currently available on Android and iOS

Q. How do I log into the app post download?

A. You can log into the app (MediBuddy) with your unique username and password. The login credentials will be same as provided for online enrolment portal).

Q. What are the benefits of using this app?

A. MediBuddy works as your one stop solution to policy and claim related information on the go. The various benefits include:

- Enrol your dependents
- Track claims status in real-time
- Locate network hospitals
- Access your ecard and other important forms and guidelines on the fly
- Access claims history and electronic health records

Q. Do I have an option to opt out of the policy?

A. Yes, you can opt out of the policy by submitting your intent in the Medi Assist website before January 31, 2026, or 30 days from the date of joining (date of joining + 29 days), whichever is earlier. The premium will not be deducted from your salary. In this case, you and your immediate family will not be covered. You will be provided only coverage for hospitalization on account of accident during the course of employment and treatment of occupational diseases, to the extent you are entitled to such benefits under any statute or law. It should be noted that owing to 2 year lock-in on the voluntary top-up coverage for the family floater, opting out in 2026 is not allowed if the coverage was availed in 2025.

Q. How is coverage activated for a new joiner?

A. Your coverage will begin from the date you have joined IBM unless you have opted out of the policy. But for your dependents, you need to visit the website (<https://portal.mediassist.in/Home.aspx>) or MediBuddy App and complete your online enrolment procedure by submitting your dependents' details within 30 days from the date of your joining (date of joining + 29 days). Please use your Talent ID@IBM as the username. An initial password has been set up for you using a combination of your date of birth and your Talent ID. For example, if your Talent ID is 123456789, your username would be **123456789@IBM** and if your date of birth is 30-November-2014, your initial password would be **30112014123456789**. Please change your password after you log in for the first time.

Premium amount due wrt insurance coverage will be prorated depending upon the Date of Joining (DOJ) of the employee. Parents-in-law cannot be covered in the policy.

Claims submitted for dependents whose name has not been enrolled in the website will not be processed by TPA.

Q. Can my sibling(s) and I include our parents under our coverage?

A. No parent can be covered twice in the policy even if he/she is a dependent of more than one employee. If two or more siblings working with IBM India are found to have enrolled their parent(s) for more than once under the policy, it will be considered as BCG violation and strict action will be taken.

Q. What if my family/dependent status change during the year?

A. Newly married employees can add spouse details within 30 days of the date of marriage. Coverage will be effective for the spouse from the date of marriage.

Employees who have a newborn child can add them within 30 days of the date of birth. Coverage will be effective for the newborn child from the date of birth.

Mid Term Inclusions (after 30 days period) will be allowed only as an exception for employees who missed adding the spouse details and newborn child details due to some valid reasons stated under MTI section above.

Any request for Midterm Inclusions for spouse and children has to have approvals from the People Manager (as per blue pages) along with the BOM Manager. The employee would need to submit proof of marriage or birth certificate whichever is applicable. The coverage date will be effective from the date the inclusion is endorsed by the insurer.

There is no midterm enhancement process for parents' policy.

There is no midterm enhancement of sum insured in respect of existing members under both the policies viz., Employee, Spouse & Children and Parents.

Claims submitted for dependents whose name has not been enrolled in the website will not be processed by the TPA.

In case of a divorce/demise in the family, please inform ibmcare@mediassist.in as soon as possible.

Also, in case of demise of a family member, please share the death certificate with date of death and send it to ibmcare@mediassist.in for them to delete the same from enrolment records.

Q. 'I am a newly hired employee. My wife has a pre-existing health condition. How is this handled/covered?'

A. Your wife's condition will be covered; there is no 9-month (waiver) period.

Q. How do I complete the Online Enrolment steps?

A. Here are the steps given below for enrolling

Step 1: Click on the link <https://portal.mediassist.in/Home.aspx>

Step 2: Enter your User ID and Password

Please use your Talent ID@IBM as the username. An initial default password has been set up for you using a combination of your date of birth and your Talent ID. For example, if your Talent ID is 123456789, your username would be **123456789@IBM** and if your date of birth is 30-November-2014, your initial password would be **30112014123456789**. (ddmmyyyy followed by empid)

Step 3: It's mandatory to change your password, before you can access any details.

Please call the support team at 080-46855351 if you face any difficulties in logging in or accessing the portal.

Note: The password is set to default at the beginning of each policy year and can be changed later by the employee as per their convenience.

i. It is mandatory to review and update Self and dependent details.

1. Go to "Your Health Policy" box on the home page.
2. Click on "Enrolment".
3. To add or edit any details please click the "Add" button or "Edit" button (as required and applicable) highlighted against each member and update the details.
4. To add dependents, click on the "Add+" button on the top right corner of the section.
5. Click "Submit" at the bottom of the page.

ii. Update bank detail to be used for claim reimbursement.

1. Go to Enrolment form.
2. Enter Bank details under the "Update Bank Details" section.
3. Click "Submit" at the bottom of the page.

2. Frequently Asked Questions (FTH Category):

Q. Who can be covered as a dependent under the FTH policies?

A. All FTH employees will automatically be enrolled under the ESC policy upon their joining / inception of the policy. They can enrol their dependents (spouse and up to 4 children) on the MediAssist Portal.

FTH employees with tenure of more than 12 months can also enrol their parents under the Parents Policy, in addition to their dependents (**spouse and up to 4 children**).

FTH employees with tenure of less than or equal to 12 months can enrol themselves along with their spouse, and up to four children till the age of 24 years. However, the policy has no age cap for a child suffering from any physical disability subject to the employee submitting the disability certificate given by competent authority.

It should be noted that the policy does not cover parents-in-law.

Q. What some insured choices are available in the policy?

A. For FTH employees with tenure of less than or equal to 12 months, the base sum insured of INR 4 lacs is available only and with no option for opting in any voluntary plans. This coverage is a floater coverage, which means, the same sum insured will be shared by the enrolled spouse and children.

For FTH employees with tenure of more than 12 months, the base sum insured of INR 4 lacs is available. The employee may choose to enhance the coverage up to INR 10 lacs with enhancement options of INR 1 lac, INR 2 lacs, INR 4 lacs, INR 6 lacs and INR 11 lacs. This coverage is a floater coverage, which means, the same sum insured will be shared by the enrolled spouse and children. Also, the employee has option to cover both their parents. The sum insured options available for Parents Policy are INR 1 lac, INR 2 lacs, INR 3 lacs, 5lacs and INR 7lacs.

Q. What does it imply if there is a 2-years lock-in period (2025 & 2026)?

A. The employees who have enrolled for the top-up/parental plan will have a lock-in for 2 years, we have introduced flexibility option of allowing employees who have enrolled for top-up/parental plan in 2025 to enhance/maintain their sum insured in 2026. Opting out of the top-up/parental plan or reducing the sum insured in 2026 is not allowed.

Q. What is an electronic ID card?

A. All employees and the insured members (provided they have not opted out), under this plan, are entitled for an e ID card. This ID card will identify them for admission into the network hospitals and to allow access to credit facilities at preferred hospitals around the country. This e ID card is non-transferable.

Note: The e ID card is the sole property of Medi Assist India TPA Private Limited and must be returned upon request or in the event of separation from the Company's services.

Q. What is the process to get an electronic ID card?

A. You can and need to print the electronic ID card online. Follow the below mentioned steps to print the ID card:

Complete the online enrolment process by adding self and dependent details by visiting Mediassist portal (<https://portal.mediassist.in/Home.aspx>) or MediBuddy App (Refer to steps for online enrolment).

It is advisable to take a print of the ID cards after completing the enrolment. These ID cards will be useful at the time of hospitalization.

Step 1: Log on to <https://portal.mediassist.in/Home.aspx> or MediBuddy APP

Step 2: Enter your User ID and Password

Step 3: You can download your e-cards anytime from the "E-cards" section of the portal or MediBuddy App

Q. What steps should I take if I lose the ID card?

A. If you lose the ID card, you may print another copy of the card online. Follow the above-mentioned steps to print copies of the ID Card.

Frequently Asked Questions (ENROLMENT)

Q. How can I check my remaining Sum Insured?

A. Please mail Medi Assist at ibmcare@mediassist.in with details about your complete name, Talent ID, Card number Or Login to Mediassist portal/app using login credentials.

Q. How should I proceed if I do not receive any revert on enrolment related queries from Medi Assist?

A. Please mail your query to ibmsupport@nivabupa.com, you shall receive revert within 48 working hours.

Q. Am I allowed to combine my group insurance with my personal insurance?

A. This cannot be done as both are different plans with different coverage.

Q. Is there an option to increase my coverage (Top-up & Parents) in 2026 by making an additional premium payment?

A. In 2026, an employee who is already part of the voluntary plan (Top UP and Parental Plan) in 2025 gets an option to increase/maintain his/her sum insured during 2026 enrolment window period (1st Jan – 31st Jan 2026). Opting for a higher coverage will only be possible during the 2026 enrolment window period and not during the mid of the policy year. However, reduction in sum insured slab or opting out under 2026 top up/parents' policy is not allowed.

Q. If I quit IBM, can I still continue to access the benefits of this policy by paying additional premium?

A. Once you leave IBM yourself and your enrolled dependents coverage under the group policy would cease / STOP effective your Last Working Day with IBM India. However, you can directly get in touch with Niva Bupa (ibmsupport@nivabupa.com) 60 days prior to your last working day at IBM India, explore the portability option and if satisfied can avail the benefits of portability for yourself and your enrolled family members (spouse and children only). You may choose to buy a retail policy with Niva Bupa, but it would be subject to underwriting with the insurer portability guidelines and retail policy will be effective post separation from IBM.

Q. Who should I reach out to for information about the products offered by Niva Bupa and to avail the portability benefit?

A. You can write to ibmsupport@nivabupa.com or megha.kaushik@nivabupa.com with following details:

- Employee Name and Talent ID
- Date of Birth
- Location
- Mobile number
- Preferred Date and time to discuss on plan options

You may choose to get in touch with Niva Bupa 60 days before your last working day at IBM India.

Q. What is the time frame to apply for applying for portability?

A. Proposal form should be submitted 60 days prior to the last working day of employment with IBM.

Q. Can I enrol my brother/sister/uncle/ aunt by paying additional premium?

A. No, this cannot be done as family definition is limited to self, spouse and 4 living children.

Q. Can I enrol my parents-in-law in the policy?

A. No, this cannot be done as family definition is limited to immediate parents. If it is identified during the policy term that parents-in-law were covered, the enrolment will be cancelled immediately.

Q. I did not enrol my parents during the 2025 enrolment window/within 30 days of my date of joining. How do I enrol my parents in the mid of the policy?

A. Mid-term addition of parents is not allowed under the policy. However, you may request for the mid-term addition under below scenarios:

- Retirement of parent where he / she was covered
 - The employee is required to submit the request to **AskHR BOT** (<https://w3.ibm.com/hr/askhr/home>) for the parents' addition within 30 days of the date of retirement
 - The employee is also required to submit relevant documents to substantiate retirement
- Demise of the bread earning parent where the other parent was covered
 - The employee is required to submit the request to **AskHR BOT** (<https://w3.ibm.com/hr/askhr/home>) for the parents' addition within 30 days of the demise of the bread earning parent
 - The employee is also required to submit the declaration of death, and a declaration that the other parent is non-working and solely dependent on the employee, and was financially dependent on the other parent

Q. If I have declared my parents in 2025, can I remove them from insurance coverage in 2026?

A. No, the parents cannot be removed from the policy in 2026, While the employees who have enrolled for the parental plan will have a lock-in for 2 years, we have introduced flexibility option of allowing employees who have enrolled for parental plan in 2025 to enhance/maintain their sum insured in 2026. Opting out of the parental plan or reducing the sum insured is not allowed.

Q. Will I be insured in the policy even if I have not got my electronic id card?

A. You and your nominated dependents will be insured from the day you join IBM/date of renewal of policy, whichever is later, provided you complete the online enrolment at <https://portal.mediassist.in/Home.aspx> or on MediBuddy mobile app within the specified timeline.

Q. What do I do if I decide to leave the services of IBM?

A. You need to inform and submit the claim (if any) to Medi Assist before your Last working day. This is very important because once you are deleted from the policy you will not be eligible to claim reimbursement for any expenses incurred.

Employees, who have resigned from IBM, **are required to respond to Shortfall raised by Medi Assist within 5 working days failing which the claim will be rejected** and prorated premium refund will be advised in FFS.

In case, there is no claim made (processed & amount settled) by you for self and nuclear family, pro-rata premium paid for coverage and/or additional coverage will be refunded back to you for the period the coverage cease to exist. In case of any claim is made (Processed & amount settled) for any one/all of the insured person, there will not be any refund of premium.

If an employee has availed the health screening benefit and undergone the comprehensive risk screening tests (all tests / screenings / vaccinations (including for Hepatitis B) under Sections A to I in the policy under Part II: Health screening benefit (beyond the Biometric screening), **there will be no refund of premium under the ESC policy.**

In case, there is no claim made (Processed & amount settled) for the parent, prorated premium paid for the concerned parent will be refunded back to you for the period the coverage ceases to exist. In case of any claim is made (Processed & amount settled), there will not be any refund of premium.

Q. What do I do in case of demise of a family member?

A. In case of demise of a family member, please share the death certificate with date of death and send it to ibmcare@mediassist.in for them to delete the same from enrolment records. Premium will be refunded (only if applicable) on prorated basis depending upon date of death and post checking the claim status. In case a claim is reported, no refund will be made.

In case of demise of one of the parents under the parents' policy floater coverage, the existing parent will continue to be under the floater coverage for the policy year and no refund will be initiated.

In case of demise of both the parents under the parents' policy floater coverage, refund will be prorated (if applicable). In case a claim is reported for either parent, no refund will be made.

Q. I enrolled one of my parents under the policy in 2025 and have submitted a claim. Now I need to increase the coverage amount. How can I do that?

A. In case your dependent parent is covered in GMC 2025 policy they will continue to be covered for the same credentials under 2026 policy (by default). However, you have an option to increase/maintain his/her sum insured during 2026 enrolment window period (1st Jan – 31st Jan 2026). The reduction of coverage amounts or removal of the parent from the policy is not allowed. It should also be noted that any ailment diagnosed / treated during coverage/enrolment window period under the lower sum insured will continue to have the lower sum insured as the maximum cover (for that ailment and all related ailments). This is applicable when the sum insured has been increased during enrolment period.

Q. I have enrolled myself, spouse and children under the ESC policy. I need to increase my sum-insured/top-up. How do I do that?

A. During the enrolment window, you may add/delete dependent and choose from the various options under the top-up table for enhancement of sum-insured. However, any

ailment diagnosed / treated during coverage/enrolment window period under the lower sum insured will continue to have the lower sum insured as the maximum cover (for that ailment and all related ailments). This is applicable when the sum insured has been increased during enrolment period.

Q. The policy mentions constant premium for 2 years (2025-2026). My parents' age will move to a higher age bracket in 2026. How do I avail the constant premium for both years?

A. Under the 2025-2026 policies, owing to the 2-year lock-in, the parents' premium will remain constant for both policy years. The parents' premium for individual coverage will be calculated basis their age as on January 1, 2025, and the sum-insured selected under the policy. The parents' premium under the floater policy (both parents being enrolled) will be calculated basis the elder parents' age and the sum insured selected. It should be noted that the premium (without tax) will remain constant for both the policy years, however, the total premium may vary in case of change in the tax rate between 2025 and 2026 or if employee is enhancing parental cover in 2026.

Q. How do I avail the option of paying parents' premium in instalments for 2025 and 2026?

A. In case your dependent parent was covered in GMC 2025 policy they will continue to be covered for the same credentials under 2026 policy (by default). If you opted for instalment option in 2025 policy, the same will be applicable for 2026 policy.

Additionally, if you are enhancing the enrolled parents sum insured in 2026 policy you will have an option to select instalment option in 2026.

The employees who did not go for the instalment option under the 2025 parental plan and/or do not enhance coverage in 2026 enrolment window shall pay the premium as a lump sum in 2025 & 2026 policy.

Q. In case of demise of one of my parents under the floater coverage, how will the coverage be treated for the other parent?

A. In case of demise of one parent under the floater coverage for 2025, the other parent will continue to be covered under the floater coverage for 2025. In case a parent from the floater coverage of 2025 passes away, the surviving parent will be moved to individual policy in 2026 with the same sum insured as opted under floater plan and the premium rate for the selected coverage under the individual policy will apply.

No refund will be processed as the other parent will continue to be under the same coverage.

3. Frequently Asked Questions (CLAIMS)

Q. Medi Assist Call Centre not giving correct update on my claim status, what should I do?

A. Please write to ibmcare@mediassist.in and seek for your claim status, in case there is no revert within 48 hrs, please write to ibm.escalation@mediassist.in and ibmsupport@nivabupa.com.

Q. What do I do in case of a hospitalization in my family?

A. In case of **planned hospitalization**, call on the helpline numbers of Medi Assist India TPA Private Limited to inform them when you or your nominated dependent needs hospitalization. This should be done at least 48 hours prior to the date of admission.

Fill the Pre-Authorization form, available with the Network Hospitals upon showing the Medi Assist ID card or by mentioning your IBM Talent ID. This can also be obtained from the Medi Assist helpline or can be downloaded from the Medi Assist website.

Submit/Fax the Pre-Authorization Form to our toll-free fax numbers at Bangalore 48 hours in advance.

The advance intimation to Medi Assist will help you to avoid payment of advance amount to some hospitals.

Sign the relevant documents including discharge summary before leaving the hospital/getting discharged. If your hospitalization is authorized, then ensure you pay for non-medical expenses and co-payment charges for the dependents and parents, if enrolled.

Emergency Cashless Hospitalization:

Pre - Auth Emergency Contact Numbers for IBM Employees - **Pre Auth-IBM: 04068178558**

When you have an emergency hospitalization and get admitted into a network hospital, please inform your family member/relative/friend to contact the billing dept in the hospital with Medi Assist ID or IBM Talent ID to send Preauthorization form to Medi Assist. If your hospitalization is authorized, then ensure you pay for non-medical expenses and co-payment charges for the dependents and parents, if enrolled. Kindly sign the relevant documents before leaving the hospital/getting discharged.

Cashless claims

Q What is Pre-Authorization?

A. Pre-authorization is a process that necessarily needs to be completed prior to hospitalization. The forms for the same can be obtained by calling Medi Assist's Telephone Help Lines or downloaded directly from the Medi Assist web site <https://portal.mediassist.in/Home.aspx> (Home Page). The form needs to be filled with the help of the treating doctor. This form contains details like details of treating physician and hospital*, details of diagnosis*, treatment proposed*, past history, estimate expenses*, signature of the treating physician*, etc. Medi Assist's medical team will then evaluate the same based on medical and policy grounds. The advance intimation to Medi Assist will help you to avoid payment of advance amount to some hospitals.

* If complete details are not provided in the form, then credit (in the case of a network hospital) or claim eligibility (in the case of an out of network hospital) cannot be provided.

Q. What is an Authorization letter?

A. On approval of preauthorization, an authorization letter will be sent to the hospital (only if it is on Niva Bupa network). The letter authorizes the hospital to extend credit for all medical expenses during hospitalization. Therefore, to use Niva Bupa cashless hospitalization service it is very important for you to follow the pre-authorization process.

Q. My Pre-Authorization request has been rejected. What could be the reasons?

A. Pre-Authorization may be declined under the following circumstances 1) Information provided was inadequate 2) Disease is not covered by policy 3) Sum insured is exhausted

Q. How to know whether a particular treatment or hospital is covered or not under insurance?

A. For list of network hospitals, please visit <https://portal.mediassist.in/Home.aspx>. In order to know whether a particular treatment is covered or not please send a pre-auth. request to Medi Assist.

Q. During my last hospitalization, I was asked for a deposit / advance. Why?

A. Network & non-network hospitals request for deposit, same is adjusted with the final bill.

Q. What do I do in case a network hospital does not accept my Medi Assist Card?

A. In case there is an issue with a network hospital not accepting your card, then please get in touch with the people mentioned in the escalation matrix (detailed below) **or call Medi Assist on 080-46855351 / 8884388455.**

Q. What is a Network Provider and how do I identify them?

A. Based on Niva Bupa's experience and expertise they have tied up with hospitals across the country so that their members can avail of cashless hospitalization facility. The list of Niva Bupa Network Hospitals is available on Medi Assist website. You may login at Mediassist portal (<https://portal.mediassist.in/Home.aspx>) or MediBuddy mobile application to make a GPS based search for the network hospitals within 20km distance of your location.

Q. Does it mean that I cannot get treated in a hospital of my choice?

A. You can get treated in any hospital within the country, but the cashless facility will be available only at the Niva Bupa network hospitals. Moreover, the collection of bills and related documents in case of a network hospital will be done by Medi Assist, whereas in case of a hospital outside of network, you will have to collect all the documents at the time of discharge and send it to Medi Assist along with a signed claim form.

Important: Please note that any hospital/nursing home you choose has to be registered and/or have minimum 15 beds.

Do I need to pay any money at the time of discharge?

A. In case of Non-Network Hospitals - You will have to make all payments yourself and then forward the claim (all the hospital documents and signed claim form in original) to Medi Assist in order to get claim reimbursed from Insurance Company.

In case of Network Hospitals - Depending upon eligibility, Medi Assist will extend credit for all the medical expenses billed by the Network hospital for the treatment of your illness only. All non-medical expenses (described above) and any amount exceeding the credit limit will have to be paid by you to the hospital at the time of discharge. In case if the patient admitted is a dependent, 80% of the medical expenses will be paid for and you will have to pay the balance 20% along with all the non-medical expenses (described above) and any amount exceeding the credit limit will have to be paid by you to the hospital at the time of discharge. The advance intimation to Medi Assist will help you to avoid payment of advance amount to some hospitals.

4. Reimbursement claims**Q. What is the process of getting rejected cases reviewed?**

A. Please mail Medi Assist at ibmcare@mediassist.in with details about your rejected claim.

Q. In case my cashless request is rejected, what should I do?

A. If your cashless is rejected due to some reason, please make the payment and submit the bills for reimbursement. The case will be reviewed as per policy terms & conditions.

Q. How do I submit my claim documents for reimbursement?

A. You can submit your claims online by **scanning the original claim documents and uploading it in the Medibuddy**. The claims can be processed on basis of soft copies and physical claim documents are not required to be shared with Medi Assist.

However, if there is any inconsistency, the insurer/TPA reserves the right to call for physical documents. In such scenarios **you can** courier the documents to Medi Assist.

Address for couriering the claim documents is:

Medi Assist India TPA Pvt Ltd;

Tower D, Fourth Floor, IBC Knowledge Park,

4/1, Bannerghatta Road,

Bangalore - 560 029

Please specify Employee Name, Talent ID and Medi Assist Health card number on the envelope.

Q. Is there an online tool to submit the claims?

A. Yes. There is a user-friendly tool for Online Medical Claim submission through the Mediassist Online Portal (<https://portal.mediassist.in/Home.aspx>).

Q. How does the tool for online medical claim submission work?

A. This feature enables you to submit both Hospitalisation and Domiciliary claims online, and upload scanned images of claim documents for faster claim processing. It will also allow you to maintain a soft copy of the claim documents submitted and stay updated on the progress with an easy and hassle-free tracking mechanism.

However, you will still be required to submit the hardcopy of financial documents in original along with the claim form and cancelled cheque to Medi Assist for further processing of the claim.

On successful claim submission, you will receive an immediate acknowledgement and claim reference number on your registered email id to access the claim details. The claim details can also be accessed on the Medi Buddy app once the claim is registered.

Online Medical Claim Submission tool offers:

1. Anywhere anytime Claim submission
2. Quick upload of claim documents

3. Secure transmission of claim details
4. Transparent and Faster claim processing /settlement
5. Real-time access to claims documents/details
6. Online submission of shortfall documents

Q. As per the doctor, admission was mandatory, then why did Medi Assist reject my claim stating as "Treatment possible on OPD basis"?

A. Claim is processed as per policy terms & conditions, after assessing the claim if it is found that admission was not required then decision will be considered accordingly.

Q. If my claim is rejected, can I ask Medi Assist to return my claim documents?

A. In case your claim is rejected for some reason; you can request Medi Assist for the original claim documents. Same will be dispatched to you within 10 working days.

Q. Hospital says all documents are given, but Medi Assist says documents not provided. What to do in such case?

A. Please check with Medi Assist what exactly are they asking for, approach the hospital with the specific requirement or please write to ibmcare@mediassistindia.com and seek for support.

Q. What is the process of availing critical illness buffer?

A. In case of specified critical illnesses, the employee is eligible for the buffer utilization as per limits specified (up to INR 700,000), please note to avail critical illness buffer, the employee has to exhaust his/her base sum insured coverage of INR 400,000/- and top-up sum insured if any.

Q. Can my claim reimbursement be credited through direct bank transfer?

A. Yes, if you declare your bank account details on the Medi Assist portal, same will be considered for direct fund transferred or you can submit cancelled cheque with name captured on the cheque while uploading the claim for reimbursement.

Q. What do I do in case my original claim documents are misplaced by me? Can I submit photocopies?

A. Claim will not be processed on photocopies as originals are mandatory. Depending upon the nature and amount of the misplaced bill, deductions will be done. However final decision vests with the insurer.

Q. If both husband and wife are working with IBM and both have enrolled each other as dependents, in that case if the maternity amount exceeds the limit, then can both claim the maternity amount?

A. Only one member can be covered once in the policy, duplication of dependent coverage is not permissible and hence maternity claim will be paid to only one employee who will be covered in the policy

Q. What is the maximum number of claims allowed in a year?

A. There is no limit on the number of claims in a year, claim amount will be limited to Sum Insured.

Q. Can I seek treatment at home and later claim it?

A. Please refer to terms and conditions under domiciliary hospitalization (detailed above).

Q. My claim has been settled but I see some deductions. What can these be?

A. Deductions can pertain to non-medical expenses, non-submission of bills, co pay deductions.

Q. If the date of admission is in 2025 and discharge is in 2026, then how will the claim be paid (if admissible)? (If the admission date is in previous plan period and discharge date is in current plan period)

A. For any claim settlement, the date of admission will decide the policy / policy period under which the claim would be considered and settled as per policy T&C and available sum insured.

5. General

Q. What is active line of treatment?

A. Active treatment is Therapeutic substance or course intended to ameliorate the basic disease problem, as opposed to supportive or palliative treatment.

Hence Active line of treatment considered when there is 24 hrs of hospitalization with therapeutic treatment or course of treatment given in the mode of IV's and etc., which reduce or cure the basic disease. Therefore, During the course of hospitalization, if there are only oral medicines with or without IM injections administered along with observations and evaluations with hospitalization more than 24hrs, then the claim would not be admissible under the policy.

However, Hospitalization primarily for Diagnostic/evaluation purposes without active line of treatment during hospitalization is covered provided the same diagnostic test/investigation done cannot be carried out in OPD.

Q. Am I eligible to claim health check-up expenses?

A. Yes, it can be claimed under OPD benefit subject to meeting the terms and conditions of the OPD benefits (if the investigation is prescribed by the Specialist than such investigation will be covered). Please refer policy t&c for more details.

Q. Is there any minimum required duration of hospitalization?

A. Yes. Stay in the hospital should be for minimum of 24 hours. However, there are a few specific ailments like Dialysis, Chemotherapy, Radiotherapy, Eye Surgery, Lithotripsy (Kidney stone removal), D&C, and Tonsillectomy which can be covered even though the period of hospitalization is less than 24 hours. For details on the same, please write to ibmcare@mediassist.in

Q. Do all day care procedures/surgeries have coverage under policy?

A. Day-care treatment strictly refers to treatments, surgeries and operations that require hospitalisation below 24hrs due to advancement in the medical technologies and procedures. Therefore, the day care procedure/surgery which is listed under Niva Bupa day care list and procedure to be done as an inpatient, then the procedure will be considered under day-care.

Q. What is a Claim Form?

A. A claim form is an important document which is essential for claim assessment. This form is provided on request by Medi Assist help desk or can be downloaded from <https://portal.mediassist.in/Home.aspx>. This form is to be signed by the member while availing the credit facility or before submission of a claim.

Please go through the reference document for claims submission published in Medi Assist micro site for IBM employees (<https://portal.mediassist.in/Home.aspx>) to minimize errors at the time of filling the form.

NOTE: We will not be able to assess your claim (credit or non-credit) without a signed claim form.

Q. What are considered Non-Medical Expenses?

A. Following are few non-medical expenses which are considered for the claim approval under the policy:

List of Non-Medical Items Payable under the policy

Serial Number	Items	Serial Number	Items
1	BABY CHARGES (UNLESS SPECIFIED/INDICATED)	40	HAIR REMOVAL CREAM

Serial Number	Items	Serial Number	Items
2	Hand wash	41	DISPOSABLES RAZORS CHARGES (for site preparations)
3	shoe cover	42	EYE PAD
4	Caps	43	EYE SHIELD
5	Cradle Charges	44	CAMERA COVER
6	Comb	45	DVD, CD CHARGES
7	EAU.DE-COLOGNE / ROOM FRESHENERS	46	GAUZE SOFT
8	foot cover	47	GAUZE
9	Gown	48	WARD AND THEATRE BOOKING CHARGES
10	Slippers	49	ARTHROSCOPY AND ENDOSCOPY INSTRUMENTS

Serial Number	Items	Serial Number	Items
11	tissue papers	50	MICROSCOPE COVER
12	Toothpaste	51	SURGICAL BLADES, HARMONIC SCALPEL, SHAVER
13	Toothbrush	52	SURGICAL DRILL
14	bed pan	53	EYE KIT
15	face mask	54	EYE DRAPE
16	flixi mask	55	X-RAY FILM
17	hand holder	56	BOYLES APPARATUS CHARGES
18	sputum cup	57	COTTON
19	Disinfectant Lotion	58	COTTON BANDAGE

Serial Number	Items	Serial Number	Items
20	Luxury Tax	59	SURGICAL TAPE
21	HVAC	60	APRON
22	housekeeping charges	61	TORNIQUET
23	air conditioner charges	62	ORTHOBUNDLE, GYNAEC BUNDLE
24	IM IV INJECTION CHARGES	63	ADMISSION/REGISTRATION CHARGES
25	CLEAN SHEET	64	HOSPITALISATION FOR EVALUATION/ DIAGNOSTIC PURPOSE
26	BLANKET/WARMER BLANKET	65	URINE CONTAINER
27	ADMISSION KIT	66	BLOOD RESERVATION CHARGES AND ANTE NATAL BOOKING CHARGES
28	DIABETIC CHART CHARGES	67	BIPAP MACHINE

Serial Number	Items	Serial Number	Items
29	DOCUMENTATION CHARGES / ADMINISTRATIVE EXPENSES	68	CPAP/ CAPD EQUIPMENT'S
30	DISCHARGE PROCEDURE CHARGES	69	INFUSION PUMP_ COST
31	DAILY CHART CHARGES	70	HYDROGEN PEROXIDE\SPIRIT\ DISINFECTANTS ETC
32	ENTRANCE PASS / VISITORS PASS CHARGES	71	NUTRITION PLANNING CHARGES - DIETICIAN CHARGES- DIET CHARGES
33	EXPENSES RELATED TO PRESCRIPTION ON DISCHARGE	72	HIV KIT
34	FILE OPENING CHARGES	73	ANTISEPTIC MOUTHWASH
35	PATIENT IDENTIFICATION BAND / NAME TAG	74	LOZENGES
36	INCIDENTAL EXPENSES / MISC. CHARGES (NOT EXPLAINED)	75	MOUTH PAINT

Serial Number	Items	Serial Number	Items
37	PULSE OXYMETER CHARGES	76	VACCINATION CHARGES
38	SCRUB SOLUTION ISTERILLIU	77	ALCOHOL SWABS
39	Glucometer & Strips	78	URINE BAG

Before a hospitalization, it is absolutely essential to follow the preauthorization procedure which will help Medi Assist to determine if the event will be covered under the policy. We understand that it would not be possible for you to remember the above-mentioned exclusions and hence the pre-authorization procedure will also help in determining the same.

Q. Will I be covered for pre-hospitalization and post hospitalization expenses?

A. Yes, you will be covered for pre-hospitalization expenses incurred 30 days prior to hospitalization and post hospitalization expenses incurred up to 60 days after hospitalization relating to the illness for which you have been hospitalized, provided that the ailment is covered under the policy. This is applicable for all eligible treatments other than maternity. NO CREDIT WILL BE OFFERED FOR THESE EXPENSES. Reimbursement of these expenses is possible only on production of complete and detailed bills and documents relating to the same along with a signed claim form.

Q. What expenses are payable as a part of pre-hospitalization and post hospitalization expenses?

A. Consultation charges prescribed medicines and prescribed investigations which are in line with the main hospitalisation diagnosis.

Q. What are details to be included in the discharge summary?

A. This is a very important document; it will mention the Date of Admission and Date of Discharge, Past History, details of treatment given, and requirement of medication post hospitalization, if any and doctor's signature. This will be on the letter head of the hospital.

Q. In how many days are claims to be submitted to Medi Assist?

A. Claim papers are to be submitted to Medi Assist Help desk representative or couriered to Medi Assist within 30 days from the date of discharge. If any further documents (information) are required then Medi Assist will send a "shortfall of document intimation" through email, and the same should be requested to hospital and submitted back to Medi Assist for further processing of the claim. In case, one does not submit the document within 3 Shortfall reminders (sent every 15 days), the case will be closed and the case cannot be processed further.

Employees, who have resigned from IBM, are required to respond to Shortfall raised by Medi Assist within 5 working days failing which the claim will be rejected and prorated premium refund will be advised in FFS.

Q. What if the charges exceed the covered amount for hospitalization?

A. In such a situation you will be liable to pay the differential amount. We will inform the hospital about your eligible amount, and they will recover the amount over and above the credit amount from you directly.

Q. How many hospital visits for treatment am I entitled to in a year?

A. There is no limit on the number of times one can take treatment, however insurance company will reimburse claims up to the sum insured. And subject to policy terms.

Q. What is the definition of a Hospital/Nursing Home?

A. Hospital/Nursing home means any institution in India established for indoor care and treatment of sickness and injuries and which

a. has been registered as a hospital or Nursing Home with the local authorities and is under the supervision of a registered and qualified medical practitioner

b. Should comply with minimum criteria as under

1. It should have at least 15 inpatient beds
2. Fully equipped operation theatre of its own wherever surgical operations are carried out
3. Fully qualified Nursing staff under its employment round the clock
4. Fully Qualified doctors should be in charge round the clock.

N.B In class C towns, condition of number of beds is reduced to 10.

Q. What is not covered under Ayurvedic treatment?

A. Ayurvedic treatment on Out-Patient basis, will not be payable. An admission for routine panchakarma treatment without establishing diagnosis is not payable. Most of the ayurvedic procedures require 1 to 2 hrs, which does not warrant hospitalization. In the absence of previous consultations, confirming Investigation reports admission for planned panchakarma and related ayurvedic treatment expenses are not payable.

Admissions at Non-registered hospitals, treatment availed in SPAs and resorts will not be payable.

Any procedure which are not a listed procedure as per the Govt Ayush guidelines would not be covered under the policy.

Q. Can I claim for Ayurvedic treatment under the cashless facility?

A. If the hospital is listed in the Insurance empanelled hospital list, then the cashless facility can be availed from that Hospital. Please check for the empanelled hospital list for the same.

Q. What is covered in Ayurvedic treatment for parents?

A. Ayurvedic treatment benefit is not covered for parents under the policy

Q. Can I claim the hospitalization expenses for abortion / abortive outcomes?

A. Miscarriage / threatened abortions / medically indicated termination of pregnancy are payable.

Hospitalization only for MTP not followed by any surgical intervention, is possible on OPD, hence not payable.

Voluntary termination of pregnancy/ multi-fetal pregnancy reductions (e.g.: triplet to twin)/and complication due to contraceptive failure are not payable under policy.

Also, Termination of pregnancy less than 12 weeks' gestation is not payable.

Q. Is Infertility treatment included in the coverage?

A. Sterility treatment /IVF treatment /Other fertility treatments are now covered under the policy up to the maternity sub-limit and can be availed twice in an employment tenure. Family planning (tubectomy, vasectomy) treatments are not payable under policy. It will however cover complications from Family Planning Devices where Hospitalization is required.

Q. Is Infertility treatment benefit covered under the basic sum-insured?

A. The Infertility treatment benefit is a part of the Maternity benefit and is covered under the maternity sub limit.

Q. Are expenses related to diagnosis/screening related to infertility treatment covered?

A. The policy does not cover any expenses made towards diagnosis, observation, screening etc. pertaining to infertility

Q. What is the maternity limit and whether any complication related to maternity pre or post to Delivery is covered under Maternity limit or General Sum Insured?

A. Maternity limit is INR 60,000 which is part of Sum insured.

Any expenses arising due to complication of maternity and directly or indirectly related to maternity would be admissible under Maternity sub limit of INR 60,000 only.

Enhanced Maternity benefits if top up is opted:

Top-up Sum Insured	Maternity Limit
100,000	65,000
200,000	70,000
400,000	75,000
600,000	80,000
11,00,000	100,000

Q. Does the Infertility treatment benefit cover OPD charges & pre/post expenses?

A. The benefit covers charges for inpatient treatment, and specified day care procedures (specified in day care list in the previous pages) only. Expenses towards OPD charges and pre/post expenses are out of scope of the benefit.

Q. ARMD, ROP, keratoconus eye complaints is payable under policy?

A. ARMD (Age related Macular Degeneration) with Injections like Avastin / Lucentis / Macugen is covered under the policy. Injection Remicade infusion is payable under policy. But C3R (CORNEAL COLLAGEN CROSSLINKING WITH RIBOFLAVIN) and INTACS are not payable. Treatment related to ROP (retinopathy of prematurity) and RFL (Retrolental fibroplasia) is not payable. For ARMD No Sublimit for ESC policy (per year), and Up to 50k for Parental policy (per year)

Q. Lasik surgery for Eye correction is covered?

A. Lasik surgery is covered if power exceeds +/-7.5. But surgeries indicated in view of cosmetic /removing contact lens / spectacles are not payable. Surgeries related to implantable contact lens not payable.

Q. Are expenses related to multifocal and toric lenses covered?

A. Expenses related to multifocal lenses and toric lenses will not be admissible under the coverage as these lenses are used only for replacement of the spectacles, and any procedure for avoiding usage of spectacles will fall under cosmetic clause of policy terms and conditions.

However, claims (cashless/ reimbursement) for Multifocal lens will be allowed if insured is under 45 years of age.

Q. Hospitalization for Investigations and evaluation is covered or not?

A. As per policy, admission into hospital for medically necessary treatment as an inpatient is payable.

The claim is admissible if the hospitalization was primarily for the purpose of investigation & evaluation (there should be requirement of hospitalisation for conducting investigation. Expense for any Investigation which are possible under OPD basis would not be payable)

Q. Under Domiciliary OPD benefit general check-ups are covered or not?

A. General check-ups / routine health check investigations, dental consultations are not payable under Domiciliary OPD benefit.

Effective 1st Jan 2025, and to continue in 2026, any screening (even prescribed by General Physician - MBBS) will be covered under the OPD plan with 50% co-pay.

Q. What expenses are not payable under OPD benefit?

A. Expenses not payable under OPD benefit are:

- a. Dental treatment related medical expenses
- b. Maternity / infertility / miscarriage / pre conception related investigation / consultations
- c. Vaccination expenses / doctor visits for vaccination consultations
- d. Routine check-ups / health check-ups
- e. Procedure charges except road traffic accidental injuries
- f. Non allopathic treatments / medical expenses
- g. Pharmacy / medicines / consumables & disposables
- h. Bills without respective consultation letters / prescriptions
- i. Consultation Bills other than specialist
- j. Consultation fees details mentioned on letter head / bills which are not in pre-printed bill formats

Q. Maternity consultations are payable under OPD benefit?

A. Not payable. Please refer Pre-natal and post-natal benefit.

Q. What do you mean by pre and post-natal expenses? Will it be covered under policy?

A. From the date of conception to delivery the expenses related to scanning's/ consultations are payable under pre-natal, after the delivery consultation expenses related to Mother will be paid up to 60 days under post-natal benefit.

Pre-natal and post-natal benefit will be payable up to INR 10000/- under maternity sub limit.

In case of medically terminated pregnancy cover pre and post-natal expenses are covered. (Only if Medical termination of pregnancy main claim is settled with us)

Q. Chemotherapy, dialysis is payable or not?

A. Payable under day care benefit. Employee has to submit Chemo / dialysis bills along with Chemotherapy chart or Dialysis chart (which explains treatment summary along treatment dates).

Q. As a Medical Advancement treatment can I claim for Stem Cell, Robotic & Bone Marrow for Cancer cases?

A. Coverage is offered subject to;

- a) the treatment having FDA approval
- b) Indications exist (specifically for Robotic surgeries - Robotic radical prostatectomy, Robotic Onco Surgery, Robotic Cardiac Surgery will be covered. However non-indicated procedures like Myomectomy etc would not be covered. In such cases usual charges of Laparoscopic surgery package charges would be payable)
- c) Bone marrow transplant for cancer would cover.

Q. Is CAPD expenses covered under the policy?

A. Expenses related to CAPD (continuous ambulatory peritoneal dialysis) Including related medicines used for CAPD are NOT payable. However, CAPD device cost is admissible under the policy.

Q. Can I claim for oral chemo drugs under hospitalization policy?

A. Yes, Oral chemotherapy drugs can be claimed under the hospitalization policy however there would be no pre and post hospitalization expenses coverage for the same.

Any expenses related to routine screening, lab test, scan etc. which is not under the pre and post hospitalization clause would not be admissible under the policy terms and conditions.

Q. Are there any Specific criteria under domiciliary outpatient benefits/OPD?

- Specialist consultations and investigations advised by specialist will be payable under the policy with 50% co-pay on actual up to 10,000 INR per family. Specialist includes
- MD, MS, DM, MCH, DGO, DNB, DCH, DPM, D Ortho, DLO, FRCS, MRCP, FRCSC & FRCAS
- Psychiatric / mental illness / suicidal treatment will be covered under OPD as per terms and conditions of the benefit
- Dental/ non-medically prescribed physiotherapy/ maternity related expenses are not payable under domiciliary benefit.

Q. Will the screening tests be payable under OPD benefit?

A. OPD benefit covers all and any screenings tests (including acute and chronic) prescribed by a General Physician. (Consultation Expenses of General physician are not payable)

Q. Is inpatient admission for administering Inj. Remicade payable?

A. As per policy Inj. Remicade administration / IV infusion is payable under hospitalization policy.

Q. RFQMR is payable or not?

A. Rotational Field Quantum Magnetic Resonance is not payable as per policy terms.

Q. What are the expenses which can be claimed related to a case of Autism?

A. Autism related expenses can be claimed under OPD benefit for possible coverage.

Specialist (Neurologists) consultation, investigation, speech therapy and occupational therapies are covered under OPD benefits with applicable co-pay for Autism related expenses.

Q. Emergency/ER related expenses covered under the policy?

A. Emergency cases/ER related expenses are covered in OPD benefit though if it is non specialist consultation as well

Q. Is All Laser treatment covered under policy?

A. Expenses related to Laser treatment Other than correction of eyesight is covered with specific indication of use of laser in place of conventional procedures by treating doctor.

Also, Laser procedures approved by FDA only covered under policy.

Q. What expenses are covered in Dog bite/Snake bite cases under policy?

A. Dogbite/Snake bite expenses like investigations and medicines or any related procedures done are covered under daycare/IP benefit if supportive day care/IP summary given as per policy.

Q. How to claim robotic charges for any surgery?

A. As per policy medically indicated robotic surgery is payable for all procedures provided if same is FDA approved.

(If any specific medical indication for choosing robotic procedure over conventional procedure other than cancer is given to us by treating doctor, then we would consider robotic expenses otherwise we would restrict up to conventional charges only as per policy).

Q. Medical Termination of pregnancy (MTP) covered under policy?

A. Hospitalization only for MTP not followed by any surgical intervention, is possible on OPD, hence not payable.

Voluntary termination of pregnancy/ multi-fetal pregnancy reductions (e.g.: triplet to twin)/and complication due to contraceptive failure are not payable under policy.

Also Termination of pregnancy less than 12 weeks gestation is not payable.

Q. What are the mandatory documents required for claim submission?**A. Checklist for employee reference:**

1. Niva Bupa claim form duly filled and signed
2. Photocopy of Ecard
3. Original medicine bills and Doctor's payment receipts with corresponding prescriptions. Doctor name and specialization should be visible in prescriptions / consultation letters.
4. Lab reports

Note: Employees do not need to submit original Xray films, ultrasound films, scans, etc. However, the employee may be asked to submit original lab reports/scans for further investigation, if required.

Additionally, based on the claim type, the employee will be required to submit the following documents:

S.No.	Activity	Details
	Claim Type	Mandatory documents for claim submission Checklist For employee reference
1	OPD / domiciliary claims	1. Treating Doctor specialization details (provided on prescription / letter head), diagnosis along with doctor sign and seal on claim form / prescriptions for all OPD claims
2	Cancer Screening Tests	1. Requisition letter (Doctor's prescription on advising specific test) 2. Original bill with details of procedure expenses 3. Investigation report (photocopy attested by the doctor) 4. Investigation bill in original
3	Inpatient / Day care claims	1. Photocopy of detailed discharge summary / day care summary with signature & seal from hospital where treatment is taken 2. Original consolidated hospital main bill with break ups of each item duly signed by insured 3. Original payment receipt of the hospital bills

		4. Original lab investigation bills with original payment receipts (if any) 5. Original invoice bills for implants (ex: stents / IOL /mesh etc) with original payment receipts along with stickers 6. In case of maternity, a certificate from treating doctor stating obstetric history with GPLA (Gravida, Para Living & Abortion) details
4	Pre and Post Hospitalization Claim	1. Original payment receipt of the hospital bills 2. Original lab investigation bills with original payment receipts (if any) 3. Photocopy of discharge summary of the main claim with signature & seal from hospital where treatment is taken
5	KYC/ AML documents if claim amount exceeds > 1 lakh; one ID proof ad one address proof has to be given mandatorily	Documents for ID proof – i. Pan Card ii. If Pan Card is not available, please submit any of the documents mentioned below stating reason for not having Pan Card. a) Passport b) Voter's Identity Card c) Driving License d) Personal Identification and Certification of the employees for your identity. e) Letter issued by Unique identification Authority of India containing details of name address and Aadhar f) Number g) Job Card issued by NREGA duly signed by an officer of the State Government Documents for address proof I. Electricity Bill not older than 6 months from the date of Insurance Contract II. Telephone Bill pertaining to any kind of telephone connection like mobile, landline, wireless etc. Provided it

		is not older than 6 months from the date of claim submission III. Ration Card IV. Valid lease agreement along with rent receipts which is not more than 3 months old as a residence proof V. Saving Bank Passbook with details of permanent/ present residence address (updated up to 1 month prior to claim submission document) VI. Statement of saving bank account with details of present/ present address (updated up to 1 month prior to claim submission document)
6	Contribution clause (If an employee is claiming from more than one insurer, the total claim expense is shared by the insurers proportionately. Each insurer contributes towards the claim payment in proportion to the sum insured limited opted.)	1. Photocopy of mandatory documents (including additional documents specified in S. No. 2) along with settlement note from other TPA / Insurer 2. Employee declaration on claimed amount from Niva Bupa and settled amount from other insurer along with balance cash paid receipts

Please go through the reference document for claims submission published in Medi Assist site for IBM employees (<https://portal.mediassist.in/Home.aspx>) to minimize errors at the time of filling the form.

Q. BPAP, CPAP is payable or not?

A. Yes. BPAP and CPAP are admissible under the inpatient benefits of the policy.

Q. I've got settlement for maternity claim under my Spouse's insurance policy from a different corporate/insurance company. Now I would like to claim for Well baby expenses and pre and post-natal expenses under IBM health insurance policy?

A. Yes. Claim pertaining to well-baby expenses up to INR 10,000 is admissible within the sublimit of maternity. Pre- and post-natal can also be payable up to INR 10,000 sublimit of Maternity.

Q. Congenital external ailments are payable under policy?

A. Cover for congenital external diseases irrespective of age is payable under policy. These expenses would be covered under the general sum insured and the same would not be covered under the critical illness buffer of the policy.

Q. Can I claim expenses related to genetic disorder?

A. Yes, expenses related to genetic disorder are admissible under the policy (the requirement of hospitalization should be established).

Q. Is Genetic test covered under policy?

A. If treatment was dependent on test results of Gene testing in case of cancer, then same is covered but Genetic testing for academic purpose or to know chances of cancer in offspring is not payable. Gene therapy is not covered.

Q. Hospitalization for CT scan or MRI scans is covered under policy?

A. As per policy admission for investigations & evaluation which requires Hospitalisation is payable.

Q. Why should I submit cash paid receipts?

A. As per policy cash paid receipt along with original bills are mandatory requirements for claim process. (cash paid receipts should contain receipt number /TIN number with the hospital name on the top and hospital seal and signature on the below. Details of expenses mentioned on the hospital letter head or doctors' letterhead are not acceptable.)

Q. Any procedure done under day care which are not listed in the policy wordings is covered or not?

A. No, the procedures which are listed in the day care list of the policy are only covered.

Q. Any medication or injection administered under day care other than chemotherapy for cancer treatment and complications are covered or not?

A. The policy will not cover any injection or any medicines other than chemotherapy drugs which is infused under the day care procedure. The expenses related to hormonal therapy / biological agents /zoledronic acid etc and other injections which are administered under day care are not payable as per policy terms and conditions. Further, for any cancer related treatment, we shall cover only the expense related to chemotherapy/radiotherapy under pre-post hospitalization /day care procedure (as may be applicable). If any day care treatment is converted to hospitalization for more than 24 hours without proper justification, the same is not payable as per policy terms and conditions.

Q. Which claims form to be used for claiming Annual Health screening?

A. OPD claim form to be used which is available in Medi Assist portal.

Q. Is there any co-payment applicable on the Annual health screening claim?

A. No, Co-payment is not applicable for Annual health check screening.

Q. Will availing the Health Screening benefit be treated as a claim?

A. Yes, it is a benefit offered as part of the policy and same will be treated as a claim if availed.

Q. Is the Health Screening claim part of Sum Insured?

A. Yes, the claim would be considered part of the General Sum Insured.

Q. What is the process of availing the Annual Health Screening benefit?

A. Combination of Onsite & offsite mode will be used. Onsite here means IBM Campus and Offsite means identified network hospitals / diagnostic centres. Employees who are not able to participate in the onsite biometric screening camps have a choice to get the screening done at identified network diagnostic centres /hospitals and file a reimbursement claim.

Q. Is the annual health screening benefit applicable for employee & Parents policy?

A. The annual health screening benefit is applicable for only employee policy.

Q. Are the expenses related to Ayurveda/Homeopathy/naturopathy under pre and post hospitalization expenses, if the main claim is settled for allopathic treatment?

A. No, the expenses related to Ayurveda/Homeopathy/naturopathy are not payable under pre and post hospitalization expenses if the main claim is settled for allopathic treatment.

Pre and post hospitalization expenses would be admissible only if the main claim is settled and the system of medicine should be the same.

For e.g., If the main claim is settled for Ayurveda treatment and the post hospitalization expenses are of allopathic treatment then the same would not be admissible. However, expense related to Ayurveda would be admissible under pre and post hospitalization expenses if the main claim is settled for Ayurveda treatment.

Q. Are expenses related to hospitalization only for physiotherapy payable?

A. Hospitalization only for physiotherapy will not be considered as an active line of treatment and the same would not be admissible under the policy terms and conditions. However Medically prescribed physiotherapy charges are payable under OPD benefit with co-pay of 50%, if specialist consultation prescribing physiotherapy given; definition of a specialist is as stated in the OPD section of the policy.

6. Frequently Asked Questions (ECASHLESS PROCESS)

1. What is eCashless?

eCashless is an offering by Medi Assist which helps you avoid wait time at the hospital's insurance desk on the day of admission to obtain pre-authorization. With eCashless, you obtain a provisional pre-authorization at the comfort of your home or office well ahead of your admission. This helps you plan your hospitalization better.

2. What are the advantages of eCashless?

eCashless has several advantages over regular cashless hospitalization. Following are a few of these advantages.

- You do not need to wait at the hospital's insurance desk for long hours
- You can obtain a provisional pre-authorization from the comfort of your home or office
- You can choose your preferred room type at the hospital
- Information regarding out-of-pocket expenses and/or co-pay can be known well in advance

All these mean better ways of planning your hospitalization.

3. Where can I avail the eCashless facility?

You can avail the eCashless facility at any of your insurer specific network hospitals.

4. When should I initiate the eCashless request?

eCashless is an offering available for planned surgeries/procedures, where you are aware of the day of admission in advance. Hence, you should initiate the eCashless request at least 48hrs prior to the day of admission.

5. How can I initiate the eCashless request?

You can initiate an eCashless request using the following ways:

- a. You can download the MediBuddy app. Go to the app and click the eCashless tile to initiate a request
- b. You can go to <https://portal.mediassist.in/Home.aspx> and click the eCashless tile to initiate the request.
- c. You can also open MediBuddy on your mobile browser and select the eCashless option
- d. You can logon to the Medi Assist portal <https://portal.mediassist.in/Home.aspx> and opt for the eCashless option
- e. Use your MAID & DOB or your MediBuddy credentials to log in

6. What are the documents I should submit for eCashless?

While initiating eCashless, you are requested to upload Doctor Consultation letter and Latest investigations reports.

7. Can I upload multiple files using MediBuddy?

- Yes. You have an option to upload multiple files when initiating the eCashless request on MediBuddy. Max file size is 5Mb.

8. What are the documents I should carry on the day of admission?

On the day of admission, you must carry the following documents:

- Secure passcode / OTP
- Medi Assist e-Card
- Photocopy of ID card of patient
- Medical advice for hospitalization or previous consultation documents
- All investigation reports (including X-ray/CT/MRI/USG/HPE)
- Any other document, relevant to the treatment of the insured

9. Under what circumstances can pre-authorization be denied?

Pre-authorization requests may be typically denied under the following circumstances:

- The ailment for which hospitalization is being sought by you is not covered under your insurance policy
- You have exhausted the eligible medical insurance cover/sum insured for the year

Note: In case of insufficient information, Medi Assist will inform you about the additional information you should provide on MediBuddy before the provisional pre-authorization is approved. In case you fail to provide this information, pre-authorization can be denied.

10. Can out-patients/health check-ups and emergency admissions avail the eCashless facility?

No, eCashless can be availed only for planned hospitalization. However, in cases where your doctor in the outpatient department recommends you to be treated as an inpatient, you can avail the eCashless facility from Mediassist portal or app.

11. How will I be updated / informed on the status of my eCashless request?

You can track the progress of your eCashless request on your MediBuddy app. You will also receive SMS notifications from Medi Assist from time to time. You may see any of the following status messages -

Additional information requested by provider - This appears when the hospital requires some more information from you on the eCashless.

Additional information requested by Medi Assist - This appears when Medi Assist requires some more information from you on the eCashless.

Denied - This appears when your eCashless request has been declined.

Approved - This appears when your eCashless request has been approved.

12. How will I come to know of my out-of-pocket expense (based on co-pay and policy conditions) while initiating the eCashless request?

When you opt for eCashless, based on your recommended treatment, the hospital sends intimation to Medi Assist regarding the expected expenses for the treatment. Once this is received, Medi Assist applies your policy terms and conditions (also available on Mediassist portal) in order to calculate the provisionally approved amount and co-pay for the treatment. This gives you complete visibility into the expected cost of the treatment.

13. Can eCashless request be cancelled if I couldn't turn up on the date of admission (DOA) or due to any other unanticipated reasons?

Yes, eCashless can be cancelled in case you do not get admitted to the hospital on the said date. However, your provisional pre-authorization is still valid for 2 weeks. This means that, if you want to get admitted to the hospital within two weeks of the said date, you can still use the provisional pre-authorization and passcode provided to you by Medi Assist. If you would like to get admitted after 2 weeks, you are expected to request eCashless again. If you wish to cancel your request, please mail ECashless@mediassist.in

7. Frequently Asked Questions (HCAM EMPLOYEES).

1. What is the IBM India Group Medical Insurance (GMC) policy for 2026?

The GMC policy provides healthcare coverage for IBM employees and their families in India. For 2026, Niva Bupa Health Insurance Company Limited is the insurer, and Medi Assist Insurance Pvt. Ltd. is the Third-Party Administrator (TPA).

2. Who is the insurer and TPA for GMC 2026?

Insurer: Niva Bupa Health Insurance Company Limited; TPA: Medi Assist Insurance TPA Pvt. Ltd.

3. What are the enrolment dates?

The enrolment window is open from December 1st to December 15th, 2025. All enrolment and payment processes must be completed within this period to ensure coverage.

4. How can I opt out of the base policy?

Employees who do not wish to continue coverage must opt out via the MediAssist portal during the enrolment window.

5. Can I opt out of voluntary plans (ESC top-up and Parental Policy)?

No, employees enrolled in voluntary plans cannot opt out due to a two-year lock-in period. You may, however, enhance your coverage for 2026.

6. How do I pay the premium if I am on a Global Assignment and not on India payroll?

You must make a direct payment of ₹2,554.42 for the ESC Base Policy to the specified IBM India account. Payments for voluntary plans must be made separately to the Niva Bupa account.

7. What are the bank account details for premium payments?

ESC Base Policy: IBM India Private Limited Bank account.

Voluntary Top-Up and Parental Policy: NIVA BUPA HEALTH INSURANCE COMPANY LIMITED Bank account.

Note: Bank account details are shared in the welcome email.

8. What should I include in the payment remarks?

Please mention your six-digit Employee ID in the payment remarks for all transactions.

9. What details should I share after making the payment?

For the base policy, email IBM with: Employee ID, Name, Amount Paid, Transaction Reference Number, UTR, and Date of Payment.

For voluntary policies, email ibmsupport@nivabupa.com with: Employee ID, Name, Amount Paid, Transaction Reference Number, UTR, Date of Payment, and Paying Bank.

Use the subject line: IBM HCAM GMC 2026 (Employee ID).

10. Who can I contact for queries?

Medi Assist: ibm.escalation@mediassist.in or ibmcare@mediassistindia.com

Niva Bupa: ibmsupport@nivabupa.com

11. What happens if I do not complete payment within the enrolment window?

Failure to complete payment within the specified dates may result in interruption of coverage for the 2026 policy year.

12. Can I enroll dependents?

Yes, you can enroll spouse & children during the enrolment window.

13. Is the premium tax-deductible?

Yes, under Section 80D of the Income Tax Act within defined limits.

14. What happens if I opt out of GMC 2026?

If you opt out, you cannot enroll dependents for 2026. You may rejoin in 2027 with a base cover of ₹4,00,000.

15. Will premiums change if parents' age increases in 2026?

No, premiums remain the same for 2025 and 2026 unless you enhance coverage.

Asked Questions (MISCELLANEOUS)

Q. How do I access the virtual chatbot at Medi Assist?

Virtual Helpdesk and Chat option on Medi Buddy portal - Steps

- Log-in to "<https://portal.mediassist.in/Home.aspx>".
- Enter your MediBuddy Username and Password.
- Click on any of the tabs reflecting in the homepage and "Talk to Us" tab will be reflecting, then click on the "Talk to Us" tab and get instant resolution to your queries.
- The user can chat with our Chat Bot.
- End chat - Close the tab to end the chat.
- Please contact [AskHR Chatbot](#) for IBM Support

Escalation Matrix:

Q. What are the Help Line Numbers?

A. For IBM employees can call on the chargeable number **8884388455 / 080-46855351**.

While travelling abroad, you can reach out to Medi Assist on the ISD Helpline number: 00 91 80 67617555

You can also connect at the following tollfree number given below:

For Pre-Auth Emergency Contact Number for IBM Employees is:

Pre Auth-IBM: 04068178558

Q. Can I email my queries to Medi Assist India TPA Private Limited?

A. Yes, there is a dedicated email id for IBM. Please send your queries to ibmcare@mediassist.in

Q. What is the escalation matrix followed at Medi Assist?

A. Following contact points are available to address employee queries.

Generic queries

1. Please call on the chargeable number **8884388455 /080-46855351**
2. Please write to ibmcare@mediassist.in on any unanswered queries by the call centre for more than 2 working days

Please follow the below mentioned escalation matrix ONLY if your queries are not replied by above mentioned contact points or the response received is not satisfactory.

Escalation Level 1	Escalation Level 2	Escalation Level 3
Please write to ibmcare@mediassist.in Turnaround Time: 1 working day	If you are not satisfied with the response received from Escalation Level 1 Please call the CRM assigned to your location or nearest location to discuss and write to ibm.escalation@mediassist.in with a copy to ibmsupport@nivabupa.com For IBM support, please write to AskHR Chatbot	In-case not satisfied with the response /non-response from Escalation level 2, please write to Khushi.singh@nivabupa.com

Note: Please write to each escalation level separately according to the timelines. Do not mark a combined mail to all the escalation points.